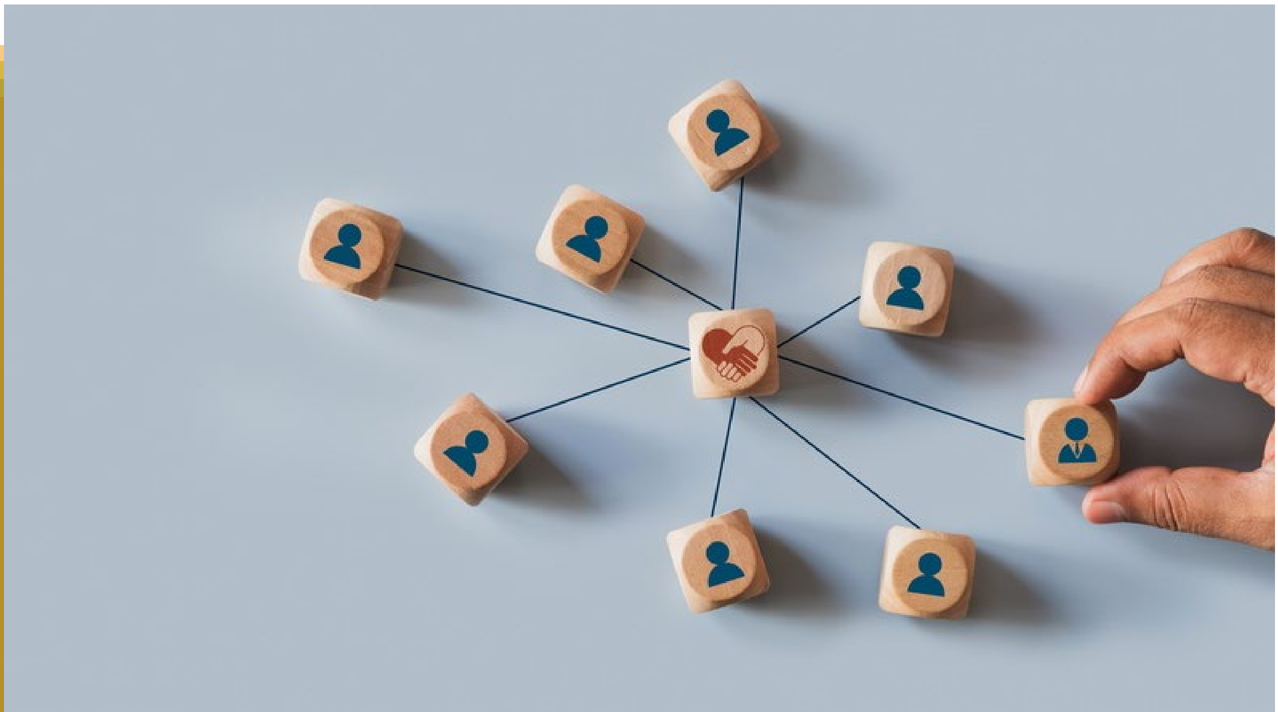
KCE REPORT 420S
HEALTH SERVICES RESEARCH

Federaal Kenniscentrum voor de Gezondheidszorg
Centre Fédéral d'Expertise des Soins de Santé
Belgian Health Care Knowledge Centre

SUPPLEMENT

MANAGEMENT OF ENCEPHALOMYELITIS/ CHRONIC FATIGUE SYNDROME IN BELGIUM: AN ANALYSIS BASED ON PATIENT NEEDS



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Reported interests KCE members:	KCE is a federal institution funded by INAMI/RIZIV, by the Federal Public Service of Health, Food chain Safety and Environment, and by the Federal Public Service of Social Security. KCE's mission is to advise policymakers on decisions relating to health care and health insurance on the basis of scientific and objective research. It is expected to identify and shed light on the best possible solutions, in the context of an accessible, high-quality health care system with due regard for growing demand and budgetary constraints. , KCE has no interest in companies (commercial or non-commercial, i.e. hospitals and universities), associations (e.g. professional associations, unions), individuals or organisations (e.g. lobby groups) that could be positively or negatively affected (financially or in any other way) by the implementation of the recommendations. All experts involved in the writing of the report or the peer-review process completed a declaration of interest form. Information on potential conflicts of interest is published in the colophon of this report. All members of the KCE Expert Team make yearly declarations of interest and further details of these are available upon request.
Reported interests:	All experts and stakeholders consulted within this report were selected because of their involvement in this topic. Therefore, by definition, each of them might have a certain degree of conflict of interest to the main topic of this report.
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Disclaimer:	• **Several external experts (cf. supra) were consulted about a (preliminary) version of the scientific report. They were selected based on their expertise in the topic of this report; potential conflicts of interest were considered in this process. Their comments were discussed during meetings and/or via email. They did not co-author the scientific report and do not necessarily agree with its content.** • **Subsequently, the KCE research team invited three independent experts ('validators', cf. supra), with no prior involvement in the study, to conduct a peer review of the final version of the scientific report. They were selected based on their expertise in the topic of this report; potential conflicts of interest were considered in this process. The validators decided whether or not the report was scientifically valid. They did not co-author the scientific report and did not necessarily all three agree with its content. Their name and affiliation are included in the colophon.** • **Finally, this report has been approved by common assent by the Executive Board.** • **KCE is exclusively responsible for the policy recommendations. Any (potential) persisting errors or omissions are the sole responsibility of KCE.**
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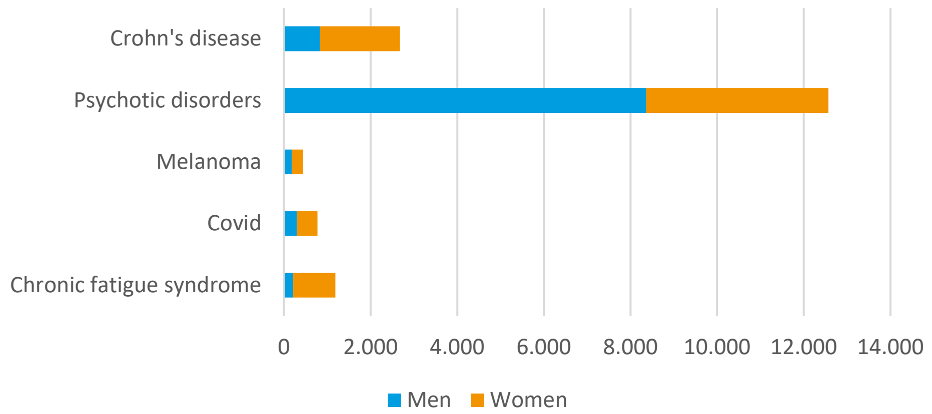
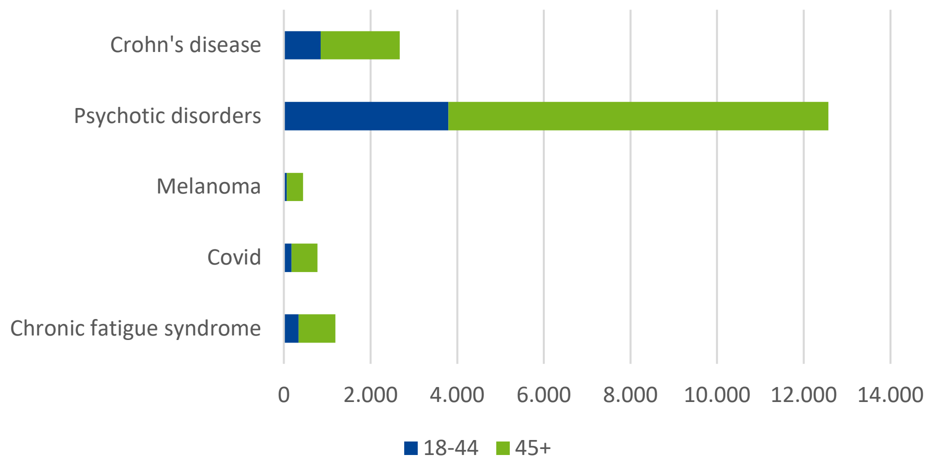
Figure 1 – Number of persons in invalidity database (men/women) on 31 December 2024



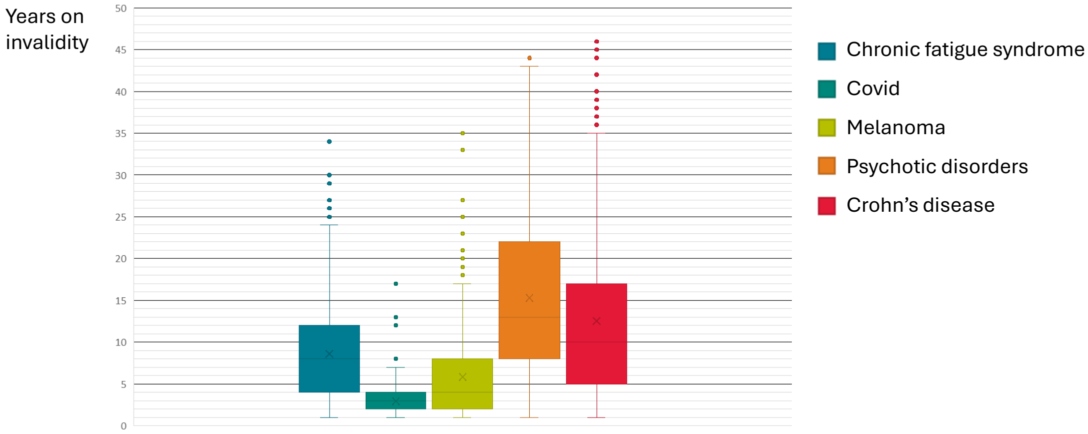
Source: 'Kenniscentrum Arbeidsongeschiktheid – Centre de connaissances en matière d'incapacité de travail' (RIZIV – INAMI)

Figure 2 – Number of persons in invalidity database (by age group) on 31 December 2024



Source: 'Kenniscentrum Arbeidsongeschiktheid – Centre de connaissances en matière d'incapacité de travail' (RIZIV – INAMI)

Figure 3 – Duration of invalidity – situation on 31 December 2024



Source: ‘Kenniscentrum Arbeidsongeschiktheid – Centre de connaissances en matière d’incapacité de travail’ (RIZIV – INAMI)

2 FRENCH AND DUTCH POSTERS USED FOR RECRUITING ELIGIBLE PARTICIPANTS


NEED
KCE
HEEFT U MYALGISCHE ENCEFALOMYELITIS, OOK GEKEND ALS CHRONISCH VERMOEIDHEIDSSYNDROOM?
• U bent 16 jaar of ouder?
• U kreeg de diagnose myalgische encefalomyelitis/ chronisch vermoeidheidssyndroom?
Neem dan deel aan onze studie, zodat wij uw zorg kunnen helpen verbeteren.
Als u niet in staat bent de vragenlijst zelf in te vullen, mag u zich laten bijstaan door een familielid of naaste hulpverlener.
CHRONIC
FATIGUE
SYNDROME
Of ga naar
https://survey.kce.be/974677?lang=nl
Deze KCE-studie maakt deel uit van het NEED-project. Dit wil onvervulde gezondheidsbehoeften van patiënten en samenleving in kaart brengen, zodat het beleid en zorginnovaties daar beter kunnen op inspelen.

3 DIFFERENCE IN EQ-5D-5L UTILITY SCORE BEFORE ONSET OF THE SYMPTOMS VERSUS TODAY BY DEMOGRAPHIC, DISEASE, HEALTHCARE AND SOCIAL VARIABLES

	Mean (SD)	Median (Q1, Q3)	Missing (n)	P-value*
Demographic variables				
Age				0.026
<30 years (n=31)	-0.41 (0.38)	-0.42 (-0.62, -0.20)	1	
30-50 years (n=292)	-0.52 (0.32)	-0.51 (-0.74, -0.28)	4	
>50 years (n=426)	-0.46 (0.35)	-0.48 (-0.71, -0.22)	15	
Sex				0.800
Male (n=72)	-0.47 (0.38)	-0.49 (-0.71, -0.22)	0	
Female (n=673)	-0.48 (0.34)	-0.49 (-0.72, -0.26)	20	
*Living situation**				0.058
Alone (n=283)	-0.51 (0.35)	-0.52 (-0.74, -0.28)	10	
With others (n=464)	-0.46 (0.33)	-0.47 (-0.70, -0.24)	10	
*Education level**				0.700
Basic/Secondary (n=271)	-0.49 (0.35)	-0.49 (-0.74, -0.26)	8	
Higher (n=473)	-0.48 (0.33)	-0.49 (-0.70, -0.25)	12	
Disease-related variables				
Time since diagnosis				0.500
<5 years (n=227)	-0.49 (0.30)	-0.49 (-0.70, -0.28)	3	
>5 years (n=522)	-0.48 (0.36)	-0.49 (-0.73, -0.24)	17	
Presence of long COVID				0.007
Yes (n=123)	-0.55 (0.33)	-0.59 (-0.77, -0.32)	1	
No (n=509)	-0.45 (0.35)	-0.45 (-0.68, -0.22)	13	
Number of unmet needs				<0.001
0-1 (n=336)	-0.38 (0.34)	-0.36 (-0.62, -0.17)	10	
2-3 (n=267)	-0.52 (0.31)	-0.52 (-0.73, -0.31)	7	
>=4 (n=146)	-0.65 (0.32)	-0.66 (-0.87, -0.46)	3	
Burden of symptoms				<0.001
Low (n=263)	-0.34 (0.31)	-0.31 (-0.53, -0.15)	6	
Medium (n=259)	-0.51 (0.31)	-0.51 (-0.70, -0.34)	6	
High (n=227)	-0.62 (0.36)	-0.67 (-0.85, -0.36)	8	
Healthcare-related variables				
Time to diagnosis				0.010
<2 years (n=297)	-0.43 (0.34)	-0.42 (-0.70, -0.23)	4	
>2 years (n=249)	-0.51 (0.34)	-0.50 (-0.73, -0.28)	6	
Satisfaction with organisation of care and planning over time				<0.001
Yes (n=288)	-0.39 (0.35)	-0.37 (-0.62, -0.17)	1	

No (n=422)	-0.56 (0.32)	-0.56 (-0.76, -0.35)	14	
Satisfaction with HCPs responsible for care coordination				0.500
Yes (n=148)	-0.51 (0.32)	-0.54 (-0.75, -0.29)	1	
No (n=490)	-0.49 (0.35)	-0.50 (-0.72, -0.26)	12	
Satisfaction with continuity of HCPs				0.600
Yes (n=586)	-0.48 (0.34)	-0.50 (-0.72, -0.25)	11	
No (n=112)	-0.46 (0.35)	-0.47 (-0.71, -0.27)	2	
Satisfaction with information received from HCPs				<0.001
Yes (n=356)	-0.43 (0.33)	-0.43 (-0.67, -0.19)	4	
No (n=363)	-0.54 (0.35)	-0.55 (-0.76, -0.30)	11	
Satisfaction with level of involvement in treatment choice				<0.001
Yes (n=430)	-0.46 (0.34)	-0.47 (-0.70, -0.24)	4	
No (n=161)	-0.57 (0.32)	-0.59 (-0.76, -0.37)	6	
Satisfaction with level of expertise of the HCPs with ME/CFS				<0.001
Low (n=416)	-0.54 (0.34)	-0.55 (-0.76, -0.30)	12	
Medium/high (n=278)	-0.42 (0.33)	-0.43 (-0.66, -0.19)	2	
Having accessibility problems				<0.001
Yes (n=316)	-0.59 (0.32)	-0.60 (-0.80, -0.38)	9	
No (n=288)	-0.37 (0.33)	-0.36 (-0.60, -0.16)	5	
Social variables				
Impact on social relationships				<0.001
Low to medium (n=190)	-0.28 (0.31)	-0.26 (-0.48, -0.10)	2	
High (n=347)	-0.49 (0.28)	-0.51 (-0.68, -0.28)	8	
Very high (n=206)	-0.66 (0.36)	-0.68 (-0.90, -0.47)	6	
Financial situation				<0.001
Easy (n=232)	-0.36 (0.32)	-0.34 (-0.57, -0.16)	5	
Hard (n=506)	-0.54 (0.34)	-0.56 (-0.76, -0.31)	15	

*Legend: * P-value is result of unpaired Student's t test (in case of 2 categories), or one-way Anova test (in case of ≥ 3 categories); HCP: health care professional*

4 DIFFERENCE BETWEEN FRENCH SPEAKING AND DUTCH SPEAKING PARTICIPANTS FOR DEMOGRAPHIC, DISEASE, HEALTHCARE AND SOCIAL VARIABLES

	French speaking (n=85)	Dutch speaking (n= 664)	P-value*
Demographic variables			
Age			0.116
<30 years (n=31)	(3) 3.5%	(28) 4.2%	
30-50 years (n=292)	(42) 49.4%	(250) 37.7%	
>50 years (n=426)	(40) 47.1%	(386) 58.1%	
Missing	0	0	
Sex			0.259
Male (n=72)	(11) 13.1%	(61) 9.2%	
Female (n=673)	(73) 86.9%	(600) 90.8%	
Missing	1	3	
Living situation			0.010
Alone (n=283)	(43) 50.6%	(240) 36.3%	
With others (n=464)	(42) 49.4%	(422) 63.7%	
Missing	0	2	
Education level			0.095
Basic/Secondary (n=271)	(24) 28.2%	(247) 37.5%	
Higher (n=473)	(61) 71.8%	(412) 62.5%	
Missing	0	5	
Disease-related variables			
Time since diagnosis			<0.001
<5 years (n=227)	(43) 50.6%	(184) 27.7%	
>5 years (n=522)	(42) 49.4%	(480) 72.3%	
Missing	0	0	
Number of unmet needs			<0.001
0-1 (n=336)	(24) 28.2%	(312) 47.0%	
2-3 (n=267)	(34) 40.0%	(233) 35.1%	
>=4 (n=146)	(27) 31.8%	(119) 17.9%	
Missing	0	0	
Burden of symptoms			0.337
Low (n=263)	(24) 28.2%	(239) 36.0%	
Medium (n=259)	(34) 40.0%	(225) 33.9%	
High (n=227)	(27) 31.8%	(200) 30.1%	
Missing	0	0	
*Change in health-related quality of life** (n=729)*	-0.57 (0.28) (n=81)	-0.47 (0.35) (n=648)	0.005
Missing	4	16	
Healthcare-related variables			
Time to diagnosis			0.352

<2 years (n=297)	(40) 59.7%	(257) 53.7%	
>2 years (n=249)	(27) 40.3%	(222) 46.3%	
Missing	18	185	
Satisfaction with organisation of care and planning over time			
Yes (n=288)	(22) 26.2%	(266) 42.5%	0.004
No (n=422)	(62) 73.8%	(360) 57.5%	
Missing	1	38	
Satisfaction with HCPs responsible for care coordination			0.055
Yes (n=148)	(11) 14.5%	(137) 24.4%	
No (n=490)	(65) 85.5%	(425) 75.6%	
Missing	9	102	
Satisfaction with continuity of HCPs			0.323
Yes (n=586)	(60) 80.0%	(526) 84.4%	
No (n=112)	(15) 20.0%	(97) 15.6%	
Missing	10	41	
Satisfaction with information received from HCPs			0.657
Yes (n=356)	(43) 51.8%	(313) 49.2%	
No (n=363)	(40) 48.2%	(323) 50.8%	
Missing	2	28	
Satisfaction with level of involvement in treatment choice			0.381
Yes (n=430)	(50) 68.5%	(380) 73.4%	
No (n=161)	(23) 31.5%	(138) 26.6%	
Missing	12	146	
Satisfaction with level of expertise of the HCPs with ME/CFS			0.460
Low (n=416)	(51) 63.8%	(365) 59.4%	
Medium/high (n=278)	(29) 36.2%	(249) 40.6%	
Missing	5	50	
Having accessibility problems			<0.001
Yes (n=316)	(49) 75.4%	(267) 49.5%	
No (n=288)	(16) 24.6%	(272) 50.5%	
Missing	20	125	
Social variables			
Impact on social relationships			0.003
Low to medium (n=190)	(14) 16.7%	(176) 26.7%	
High (n=347)	(34) 40.5%	(313) 47.5%	
Very high (n=206)	(36) 42.9%	(170) 25.8%	
Missing	1	5	
Financial situation			0.691
Easy (n=232)	(28) 33.3%	(204) 31.2%	
Hard (n=506)	(56) 66.7%	(450) 68.8%	
Missing	1	10	

Difference in working status			0.346
Stopped working (n=387)	(41) 75.9%	(346) 69.8%	
Did not stop working (n=163)	(13) 24.1%	(150) 30.2%	
Missing	31	168	

*Legend: * Based on Chi-square test or Fisher's exact test, **Based on change in utility score (EQ-5D-5L) from before first symptoms of ME/CFS to the moment of survey completion, with higher negative values reflecting larger declines, presented values are mean (standard deviation), analysed with unpaired Student's t-test*
HCP: health care professional

5 DIFFERENCE IN CURRENT EMPLOYMENT STATUS BEFORE ONSET OF THE SYMPTOMS VERSUS TODAY BY DEMOGRAPHIC, DISEASE, HEALTHCARE AND SOCIAL VARIABLES

	Still working (n=163)*	Stopped working (n=387)	Missing(n) **	P-value***
Demographic variables				
Age				0.006
<30 years (n=31)	(6) 37.5%	(10) 62.5%	15	
30-50 years (n=292)	(82) 36.4%	(143) 63.6%	67	
>50 years (n=426)	(75) 24.3%	(234) 75.7%	117	
Sex				0.245
Male (n=72)	(11) 22.4%	(38) 77.6%	23	
Female (n=673)	(152) 30.4%	(348) 69.6%	173	
Living situation				0.723
Alone (n=283)	(63) 30.6%	(143) 69.4%	77	
With others (n=464)	(100) 29.2%	(243) 70.8%	121	
Education level				0.361
Basic/Secondary (n=271)	(51) 26.2%	(144) 73.8%	76	
Higher (n=473)	(112) 31.8%	(240) 68.2%	121	
Disease-related variables				
Time since diagnosis				0.057
<5 years (n=227)	(57) 35.4%	(104) 64.6%	66	
>5 years (n=522)	(106) 27.2%	(283) 72.8%	133	
Number of unmet needs				0.797
0-1 (n=336)	(76) 30.6%	(172) 69.4%	88	
2-3 (n=267)	(60) 29.7%	(142) 70.3%	65	
>=4 (n=146)	(27) 27.0%	(73) 73.0%	46	
Burden of symptoms				0.013
Low (n=263)	(68) 35.8%	(122) 64.2%	73	
Medium (n=259)	(57) 30.6%	(129) 69.4%	73	
High (n=227)	(38) 21.8%	(136) 78.2%	53	
Healthcare-related variables				
Time to diagnosis				0.027
<2 years (n=297)	(73) 34.6%	(138) 65.4%	86	
>2 years (n=249)	(43) 24.3%	(134) 75.7%	72	
Satisfaction with organisation of care and planning over time				0.364
Yes (n=288)	(68) 31.1%	(151) 68.9%	69	
No (n=422)	(85) 27.4%	(225) 72.6%	112	
Satisfaction with HCPs responsible for care coordination				0.901

Yes (n=148) No (n=490)	(33) 28.7% (109) 29.3%	(82) 71.3% (263) 70.7%	33 118	
Satisfaction with continuity of HCPs Yes (n=586) No (n=112)	(131) 30.1% (24) 29.3%	(304) 69.9% (58) 70.7%	151 30	0.878
Satisfaction with information received from HCPs Yes (n=356) No (n=363)	(81) 30.9% (78) 29.2%	(181) 69.1% (189) 70.8%	94 96	0.669
Satisfaction with level of involvement in treatment choice Yes (n=430) No (n=161)	(96) 30.5% (35) 28.7%	(219) 69.5% (87) 71.3%	115 39	0.714
Satisfaction with level of expertise of the HCPs with ME/CFS *Low (n=416)* *Medium/high (n=278)*	(79) 26.2% (71) 33.6%	(223) 73.8% (140) 66.4%	114 67	0.066
Having accessibility problems *Yes (n=316)* *No (n=288)*	(52) 23.2% (81) 36.8%	(172) 76.8% (139) 63.2%	92 68	0.002
Social variables				
Impact on social relationships *Low to medium (n=190)* *High (n=347)* *Very high (n=206)*	(63) 44.7% (74) 28.4% (25) 17.1%	(78) 55.3% (187) 71.6% (121) 82.9%	49 86 60	<0.001
Financial situation *Easy (n=232)* *Hard (n=506)*	(66) 39.8% (97) 25.6%	(100) 60.2% (282) 74.4%	66 127	<0.001

*Legend: *Still working includes responses of "no impact of ME/CFS on work" and "working less because of ME/CFS"; ** Including answers "I don't know", *** Based on Chi-square test or Fisher's exact test*
HCP: health care professional

6 ANALYSIS OF THE OPEN QUESTION OF THE SURVEY

KEY POINTS TABLE

- Recognition as a central need for people with ME/CFS
- Recognition is needed for ME/CFS at several levels i.e. healthcare providers, policy, society.
- The lack of recognition contributes to misunderstanding, misrecognition and stigma.
- This is enforced by contributing factors such as psychologisation of the disease, and the lack of training, expertise, and scientific research.
- This brings along several consequences related to healthcare and the biopsychosocial system of the person.
- Concerning healthcare related consequences, the impact on diagnosis, available therapies and accessibility to appropriate care were identified.
- Concerning the biopsychosocial system, the impact on activities of daily living, quality of life, work and employment, the financial system and the psychosocial impact were highlighted.

6.1 Background and methods

At the conclusion of the survey, participants were invited to respond to an optional free-text question (Q68) asking whether they wished to add any additional needs not addressed in the survey (Table 1). All responses (n = 237) to the free-text question were extracted and compiled into a dataset for qualitative analysis (NVivo®) to facilitate systematic coding and organisation. Initial familiarisation with the data was achieved through repeated reading of all responses. An inductive thematic analysis was conducted following principles of qualitative content analysis. A coding tree was developed in a data-driven manner, with codes generated to reflect recurring ideas, concepts, and patterns identified in the text. The coding framework was continuously refined during the analysis process, with new codes added and existing codes modified. This process preserved the original context and intent of participants' statements. Representative quotes were retained to support and illustrate identified themes, ensuring transparency and grounding interpretations in the data. To enhance readability, the quotes were translated in English by means of an AI agent.

Table 1 – Open question in survey in Dutch and French

Questions in Dutch	**Questions in French**
Q67 - Wilt u ons vertellen over één of meer andere belangrijke behoeften die niet aan bod kwamen in de vragenlijst? * ***Kies één van de volgende mogelijkheden: Ja / Nee***	Q67 - Souhaitez-vous nous faire part d'un ou de plusieurs autres besoins importants que vous n'avez pas pu exprimer dans le questionnaire ? * *Veuillez sélectionner une seule des propositions suivantes* : Oui / Non
Q68 - Welke behoeften werden niet behandeld in de vragenlijst? ***Beantwoord deze vraag alleen als aan de volgende voorwaarden is voldaan: Antwoord was 'Ja' bij vraag ' [G1]' (Wilt u ons vertellen over één of meer andere belangrijke behoeften die niet aan bod kwamen in de vragenlijst?)*** ***Vul uw antwoord hier in: [Free text box]***	Q68 - Quels besoins n'ont pas été abordés dans le questionnaire ? Répondre à cette question seulement si les conditions suivantes sont réunies : La réponse était 'Oui' à la question ' [G1]' (Souhaitez-vous nous faire part d'un ou de plusieurs autres besoins importants que vous n'avez pas pu exprimer dans le questionnaire ?) *Veuillez écrire votre réponse ici :* [Free text box]

6.2 Recognition as a central need for people with ME/CFS

Across the responses, recognition emerged as the overarching need shaping participants' experiences and needs with healthcare, social systems, work, and daily life. Respondents described how insufficient recognition of ME/CFS as a serious biomedical condition leads, at many levels, to misunderstanding, stigma, and barriers to support.

6.2.1 Lack of understanding, misrecognition and stigma of healthcare professionals and in society

The respondents described ME/CFS as an '*invisible illness*', expressing that their **symptoms are little acknowledged** by (medical) healthcare professionals and society in general. This invisibility is coupled with trivialisation, illustrated by expressions such as '*You look well, so it can't be that bad'*.

> *"CFS is often very invisible, and there is still a lot of misunderstanding. When you go outside, you look completely normal; nothing shows on the outside. (And I'm also happy and in a good mood when I'm 'out among people' for a moment.) But the fact that, for example, you spent the whole day lying on the couch beforehand isn't visible. That makes CFS hard for others to grasp and leads to misunderstanding. I hear people justifying themselves, emphasizing all the things they can't do or saying 'I'm having a better period now'. How do you explain what CFS is? How do you get understanding from your environment for your situation? How do you find the balance between creating understanding by explaining it, without feeling like you're justifying yourself? And above all, how much energy do you put into that or not?"*

Respondents referred to common **stereotypes applied by society**, such as ME/CFS being dismissed as a '*housewife's disease'*, or a condition primarily defined by *'tiredness'*. They feel it carries **judgments and prejudices** from people even within one's close circle. They feel the need to constantly justify their behavior. They explained the **stigma** feels more traumatising than the loss of quality of life, because of the lack of understanding, and even disbelief, from family and friends. They feel the stigma is reinforced by healthcare providers, the media, and policy.

> *"I am told that ME/CFS and long COVID are trendy illnesses that people don't believe in."*

> *"The biggest problem for the outside world (including family members who don't live with you) is that on different days, or even hours, you are capable of different things. One day (or hour) you can do something, and later you can't – or the other way around. That is then seen as proof that you are exaggerating or faking it. That lack of understanding was more painful to me than my worst pain attacks. The closer the person who doesn't believe you is to you, the more it hurts."*

> *"There is also far too little understanding asked of society. None. 'You look good.' As if you have to look shabby and crumpled to be ill. When I see those campaigns for cancer awareness, I actually feel discriminated against."*

Participants described that **misunderstanding and stigma** surrounding ME/CFS profoundly shaped their interactions with **healthcare professionals**, **influencing communication**, access to appropriate care, and overall trust in the medical system. Many recounted experiences of being dismissed, disbelieved, or having their symptoms attributed to psychological causes, which reinforced feelings of frustration and isolation. This lack of recognition leads to avoidance of medical consultations and a general reluctance to disclose their diagnosis.

> *"CFS is an exclusion diagnosis, and you are really left on your own living with it. You don't go to doctors anymore, because they themselves admit they don't know what to do."*

> *"I had a doctor telling me I just needed to go back to work and stop being weak. When I insisted that I was too ill, he said he could not help me because he didn't believe in the disease."*

> *"ME/CFS has been psychiatrised for years and still is. In important medical exams, I hide my previous ME/CFS diagnosis because I have repeatedly felt and experienced not being taken seriously once ME/CFS is mentioned."*

“In reality, patients should be able to talk openly about CFS, but that is not the case. Because of that, they also cannot be properly helped.”

They stated that **the fluctuating presentation of symptoms** associated with the disease may contribute to misunderstanding, underdiagnosis and even incorrect treatments.

6.2.2 Lack of understanding and misrecognition in institutions and policy

Participants consistently reported a profound **lack of institutional understanding and recognition** of ME/CFS across certain social services, insurance bodies, etc. They described interactions with these institutions as burdensome, adversarial, and at times humiliating, characterised by excessive administrative demands, unexplained refusals of benefits, and assessments conducted by professionals lacking expertise in ME/CFS.

“How inhumanely you are treated as a sick person during evaluations (for example, by the advising physician at the health insurance fund).”

Navigating administrative and insurance procedures was described as **complex and emotionally exhausting**. Several participants reported delays or denials of financial assistance, coupled with stigmatizing attitudes from institutions, employers, or even family members, who sometimes viewed them as ‘*profiteers*’ of the system.

Participants recounted being **denied disability allowances or social protection** despite having an established diagnosis, leaving them without financial security. They expressed urgent needs for improved expertise among disability evaluators, fair and transparent assessment procedures, and governmental recognition of ME/CFS.

“It’s outrageous: despite my established diagnosis, the Disability Service did not recognize me! So I have no income at all!”

Participants described considerable **psychosocial strain** arising from both the direct effects of ME/CFS and from social and institutional responses to the illness. A recurrent theme was the experience of not being taken seriously by medical professionals and administrative bodies, which generated feelings of **abandonment and exacerbated emotional distress**. Participants reported that disability procedures and other bureaucratic processes were exhausting and deleterious to health, and that uncertainty about political decisions regarding benefits amplified their stress.

The resulting **financial strain** was described as severe, often preventing patients from accessing necessary daily support. Respondents highlighted **the importance of support for family members and informal caregivers**, particularly given the high degree of dependency created by the illness. They called for formal recognition and financial support for such arrangements.

“Hiring paid help isn’t possible because of my limited financial means.”

“Over the years my symptoms gradually became worse and more wide-ranging (disabled since 2013), which means my need for assistance is now very severe. My mother and housemate (75 years old) are also my informal caregivers, and I obviously feel very bad about that.”

Respondents also described major **challenges accessing disability support**, pointing to lengthy, complex procedures and assessment tools that do not reflect the fluctuating nature of ME/CFS or its non-visible limitations. **Disability rating scales** were described as overly focused on overt physical limitations, with little consideration for symptom variability or the cumulative impact of activity. For example, the ability to stand or walk briefly (even if followed by prolonged recovery) was sufficient to disqualify individuals from receiving support they strongly perceived as necessary, such as **disability parking cards**. While many patients evoke the need for a disability parking card, most indicate to not receive it.

“Due to the limitations that CFS brings, I have been granted disabled status. This is based on a points system. However, since I can still stand up independently and, at certain times of the day,

walk for 10 minutes, I receive only 1 point for mobility. As a result, I do not qualify for a disability parking card, even though I really do need one."

"The criteria for a disability parking card are based on an obvious physical disability and not on the delayed impact after movements or exertion."

Participants articulated the need for **a centralised, authoritative platform providing information on reimbursement pathways, available services, and clinical expertise in ME/CFS**. They also stressed the need for a specific **service or care coordinator** familiar with the multisystemic nature of the illness. Moreover, they suggested improvements including public awareness campaigns, clear political commitments, and standardised, ME/CFS-informed evaluation tools that reflect the illness's fluctuating trajectory.

"GPs are well-meaning, but not well-informed. I need a place where I can go with all my medical problems."

6.3 Identified contributing factors to misunderstanding, misrecognition and stigma

Across the responses, the psychologisation of MS/CFS with the absence of a biomarker for diagnostic, lack of knowledge and training for healthcare professionals, little evidence-based treatments and lack of scientific evidence were seen as contributing factors to misunderstanding, misrecognition and stigma.

6.3.1 *Psychologisation of ME/CFS*

Participants emphasised that ME/CFS is still frequently **framed as psychological rather than biological**. They feel that psychologisation contributes to mismanagement and delayed diagnosis reinforcing stigma and undermining the perceived legitimacy of their condition.

Participants are in need of a broad recognition that ME/CFS is a severe, complex multisystem disease that cannot be explained psychologically, but is indeed a biological illness. They express, it is currently placed under psychiatry and categorised as '*between the ears*' or '*in the head*'. Therefore, participants feel that a biological marker is needed that proves the illness.

"Raising awareness among healthcare professionals is more than necessary. This awareness is also needed within social security, health insurance funds, and insurance companies. I recently encountered a medical questionnaire for an insurance product that categorised chronic fatigue syndrome (without any mention of ME or ME/CFS) and fibromyalgia, which I also suffer from, under the heading of mental disorders. This upset me again, given previous encounters with healthcare professionals who saw my symptoms as simply 'in my head'."

Participants feel that political, institutional, scientific, and educational improvements are needed for ME/CFS to be understood as **a legitimate biological illness**.

6.3.2 *Lack of knowledge, training and expertise*

Participants indicate to have become **their own doctors** because of the lack of knowledge and education among (medical) healthcare practitioners.

"I repeatedly had to explain to my GP what CFS is, what the symptoms are, and how it affects me. A complete reversal of what should happen in the doctor-patient relationship."

6.3.2.1 *Education and training of healthcare professionals*

Respondents linked lack of recognition directly to **insufficient professional training**. They feel that medical education does not adequately cover ME/CFS or related post-infectious syndromes (such as long COVID, Epstein-bar infection, etc.), resulting in variable and often **inadequate care**. They explained that understanding of the disease by medical specialists is poor or dismissed, and patients have to point to scientific studies themselves to highlight their needs. They feel the support and knowledge are arbitrary depending on the doctor, creating uncertainty. They expressed the need that outdated knowledge must urgently be addressed in the training of all healthcare professionals. Participants expressed frustration that much of their care depended on individual clinicians' personal interest rather than standardised expertise and evidence-based care.

> *"Doctors should be trained in post-infectious syndromes and stop treating patients as hypochondriacs, showing empathy and humility regarding their lack of knowledge."*

> *"My general practitioner does what he can, but his knowledge of ME/CFS is limited, although he is the only one who has an overview of all my symptoms. The number of times you encounter a wall of misunderstanding with specialists is impossible to count. It's also frustrating that you have to see different specialists for different ailments, even though they are all connected to ME/CFS."*

The participants feel that proper training for **first-line healthcare providers**, such as general practitioners, is needed so they can correctly diagnose and understand the illness. In particular, they stated that all healthcare practitioners need to understand PEM, but also the related comorbidities such as Postural Orthostatic Tachycardia Syndrome (POTS), small fiber neuropathy, etc. They feel this knowledge is currently largely lacking, which means patients are not only treated incorrectly; or, denied any care; but also feel they have to push themselves beyond their limits just to receive treatment.

> *"I completed a cognitive behavioral therapy program of about 15 sessions, but I didn't really feel that the treating psychologist was an expert in ME/CFS. Personally, I also felt there was a lack of expertise in ADD (attention deficit disorder), and there was no discussion about possible links or triggers between ME/CFS and ADD. "*

6.3.2.2 Scientific research and evidence-based care

Alongside training, participants emphasised that ME/CFS remains one of the most '*neglected diseases*' in **scientific research**. They expressed the need for biomedical studies, improved diagnostic tools, and potential treatments, including research into off-label medications. They also described the need for a biological marker to solidify the legitimacy of the disease.

> *"There is a need for broad recognition that ME/CFS is a severe, complex multisystem disease that cannot be explained psychologically […] a biological marker is needed that proves the illness."*

They turned to the internet (while recognising the risk of misinformation) due to the absence of accessible, knowledgeable specialists. This further reinforced calls for recognised medical experts and evidence-based guidance to prevent reliance on unverified or exploitative practices. Participants link the absence of research to **the lack of available evidence-based therapies** and treatments.

> *"And the government? For years, the response was, 'We follow the NICE guidelines,' whenever patients advocated for ME. The guidelines have finally been updated, and now Belgium no longer follows them?"*

6.4 Identified consequences of misunderstanding, misrecognition and stigma

Misunderstanding, misrecognition and stigma surrounding ME/CFS were described as having wide-ranging implications for communication, accessibility to care, diagnosis, treatment, etc.

6.4.1 Healthcare related consequences

6.4.1.1 Impact on diagnosing ME/CFS

Participants reported substantial **challenges in obtaining an accurate diagnosis of ME/CFS**, largely due to limited clinical expertise and persistent misconceptions within healthcare settings. They described receiving psychiatric labels instead of appropriate differential diagnosis, particularly in places where ME/CFS remains poorly recognised. This contributed to feelings of disbelief and added an additional layer of struggle on top of the illness itself.

> *"General practitioners, psychiatrists, physiotherapists, and psychologists have little or no knowledge of CFS and want to treat you as if you are depressed, which I am not. I now present a medical certificate to doctors stating that since treatment with that physician (7 years ago) I have not had depression, in order to be treated normally and not as a depressive patient"*

Respondents emphasised the need for **clearer differentiation** between ME/CFS and psychiatric disorders, noting that commonly used tools such as depression screening instruments or the BelRAI do not account for hallmark symptoms like PEM, POTS, sensory hypersensitivities, or orthostatic intolerance. As a result, **misclassification and inappropriate treatments** were frequently experienced. They evoked the need for early systematic clinical assessment, faster referral by the GP, and consequently quicker diagnosis of ME/CFS as earlier recognition contributes to faster recovery and especially prevent further deterioration.

> *"With psychiatrists and psychologists, better differentiation between depression and CFS is needed: even without depression, as a CFS patient you can very easily score high on depression screening instruments due to the content and wording of the questions. This can lead to misdiagnosis, heavy medication that does not work, and a delayed CFS diagnosis as a result!"*

The importance of **expert-led diagnosis** was consistently stressed. Respondents expressed concern that diagnoses by non-specialists or self-diagnosis reinforce stigma and perceptions of '*fake patients'* and **called for recognised reference centers with expertise in both diagnosis and treatment**. Participants also pointed to gaps in diagnosing ME/CFS in children and noted that viral triggers (e.g., Epstein–Barr virus) should be more systematically considered.

A recurrent theme was the lack of reliable, **evidence-based information** at the time of diagnosis. Participants reported feeling abandoned and left to navigate treatment options on their own, often resorting to extensive self-directed research.

> *"I received the diagnosis… and that was it. I am exhausting myself searching for information – this already used up all my energy."*

6.4.1.2 Impact on available therapies and (evidence-based) treatments

Participants described substantial **shortcomings in available treatment options** for ME/CFS, which they attributed to limited clinical expertise, ongoing stigma, and persistent misrecognition of the illness. They reported receiving no meaningful treatment after diagnosis and were told to '*learn to live with it*'. Participants felt that fluctuating symptoms were poorly understood, leading to inappropriate prescriptions or clinical decisions.

> *"Never any treatment was considered, and any attempt to discuss it with other doctors was met with mockery, because it is supposedly just a catch-all term with no medical or scientific basis to support the reality of the illness. So no treatment or care, and not even any research on this potential diagnosis"*

They emphasised that even in the absence of curative treatments, **recognition itself** through knowledgeable clinicians, validated pathways, and clear referral networks is essential to restoring trust and equity in healthcare delivery.

> *"Need for an independent database/overview of doctors, specialists, healthcare providers, … who take ME/CFS seriously. Even if they do not have an accurate treatment, the recognition itself is already important."*

A dominant theme concerned **the use of certain (non-evidence-based) treatments**, such as cognitive behavioral therapy and graded exercise therapy. Especially the latter was experienced as harmful, particularly when PEM was not taken into account. Participants emphasised that exercise-based therapies worsened symptoms and expressed frustration that physiotherapy frequently prioritised training rather than supportive or relaxation-focused approaches.

> *"To my knowledge, it is the only illness that responds negatively and dangerously to physical treatment. We absolutely must stop saying that exercise is good for everyone!"*

> *"The standard therapy was physiotherapy aimed at improving physical fitness. No matter how often I said that physiotherapy three times a week for half an hour was too much (in the end I went twice a week), that daily home exercise on the stationary bike was impossible, and that after that half hour of effort at the physio I needed days (actually until the next session) to recover (so lying flat and unable to do much—no washing, no dressing, barely enough strength to sit at the table), and that I was worse off than without physio (because then I did have the energy and strength to wash/dress myself), they simply did not listen"*

The absence of a **structured care trajectory** was repeatedly noted. Respondents highlighted that treatments, when offered, were often delayed, required significant effort that triggered PEM e.g. by the lack of coordination referring them to several healthcare providers, and ended abruptly without follow-up, leaving them feeling abandoned.

> *"Getting a diagnosis took a very long time. But it is especially afterwards that you end up in a black hole. After 1.5 years of guidance through a multidisciplinary center, the treatment simply stops and there is no further follow-up."*

Participants stressed the need for **multidisciplinary, coordinated care involving specialists trained in ME/CFS**, with recognition of complex sensitivities to medications, additives, and therapeutic stimuli. They also noted that current practices lag behind scientific developments, especially post long-COVID insights.

> *"That doctors look at the body as a whole, rather than having one doctor for urology, another for the digestive system, etc., and if this is not possible, a multidisciplinary approach where doctors actually communicate with each other and search for a possible treatment."*

> *"Getting treatment is sometimes so complicated that it actually worsens the illness, because we have to exceed our energy limits (travel, administrative tasks, trying new treatments…). All of this triggers post-exertional malaise and worsens my symptoms."*

Participants reported difficulties accessing supplements, specialised tests, or extended physiotherapy sessions creating **financial barriers**. The respondents feel that patients lose time and money just trying to find medically trained professionals who even acknowledge their condition. They risk ending up in the alternative health circuit, where certain 'practitioners' may take advantage of vulnerable people and charge a lot of money for non-licensed therapies. They advocated for **aligned reference centers and a registry of effective patient-reported treatments**.

6.4.1.3 Impact on accessibility and appropriateness of care

Participants described substantial **barriers to accessing appropriate healthcare for ME/CFS**, closely linked to limited clinical knowledge, fragmented services, and environments poorly adapted to the needs of patients with severe or fluctuating symptoms. Participants reported falling through eligibility criteria for municipal transport assistance because their ability to walk or drive varied unpredictably. This uncertainty, combined with fear of judgment, discouraged the use of such services.

> *"Sometimes I can manage short distances, but I never know in advance when I'll collapse from acute exhaustion—half an hour before an appointment, I might completely shut down and need to lie down for two hours. Since I can still walk upright and drive in between, it feels awkward to contact such services."*

They reported that the unpredictability of their condition made it difficult or impossible to attend scheduled appointments, while the healthcare system provided few alternatives such as **home visits or teleconsultations**.

> *"When you are severely ill, you can't even reach the doctor because of the unpredictability of symptoms. Teleconsultations and home visits should be standard."*

Referral pathways to ME/CFS-trained professionals were suggested to reduce emotional burden, exertion and prevent relapses.

> *"The care system often feels like a barrier rather than a help. Besides being extremely disabling, this illness is mentally draining because you realize there's nowhere you can turn."*

Participants emphasised that speaking itself can be fatiguing, underscoring the need for **adapted communication approaches** in clinical encounters. Next to that, they highlighted the lack of low-stimulation, physically **accommodating spaces in healthcare settings** e.g. quiet, dark and low stimulus waiting rooms or the possibility to lie down during appointments, quiet single rooms in hospitals and staff trained to communicate calmly and avoid sensory triggers, which rendered routine care inaccessible for those with orthostatic intolerance or sensory sensitivities.

> *"There should be quiet, dark spaces in hospitals for people who cannot sit upright for long periods. Otherwise, we are excluded from care altogether."*

> *"Requesting a place to lie down is always met with difficulty… I often end up lying on the floor while waiting."*

Experiences within existing reference centers were mixed. While some appreciated initial information sessions and peer contact, others described feeling unheard when reporting deterioration under prescribed therapies. Limited availability of knowledgeable clinicians and **long waiting** times intensified these difficulties. Reference centers were perceived as **scarce, overburdened, and inconsistently aligned with current scientific insights**.

6.4.2 Biopsychosocial consequences

6.4.2.1 Impact on activities of daily living and quality of life

Participants described a marked **decline in daily functioning and quality of life**, with even basic household tasks becoming exhausting or impossible. The continual need to manage fluctuating symptoms and pacing contributed to a persistent sense of overload.

> *"My quality of life has drastically decreased! Staying up late is a disaster, doing anything extra is a disaster — you name it, my life is nothing like it once was. Without my daughter, life would no longer be worth living for me this way."*

Activities outside the home were often unfeasible, and some participants proposed practical measures such as identification cards to ease public interactions during relapses. Cognitive impairments were also reported as severely limiting daily functioning and participation in research or therapy.

> *"On the next page of this survey, I see the question about an interview. I would have liked to do that, but unfortunately my cognitive limitations, among other things, are too severe for this. That's why I answered 'no'."*

Participants described substantial unmet needs related to information, practical support, and environmental adaptations required to **participate in daily life** and broader society. Many emphasised **structural barriers** in the built environment including inaccessible restaurants, narrow doorways, and non-adapted stairways, which restricted mobility and social participation. In line with this, respondents highlighted the need for **adapted housing** and cohousing options designed for low-stimulation living, noting that financial constraints and limited availability of social housing made it difficult to obtain suitable accommodation.

> *"I have been looking for a quiet place to live since after the pandemic—already for 4 years."*

6.4.2.2 Work, employment and financial impact

Participants described significant challenges related to work capacity and financial stability due to ME/CFS. While many expressed **a strong desire to remain professionally active**, the unpredictable and debilitating nature of their symptoms made full-time or even part-time employment extremely difficult. This situation created what participants referred to as a "*double burden*" of illness and financial insecurity. Existing systems for disability benefits, partial work accommodations, and reintegration were perceived as rigid, fragmented, and poorly adapted to the fluctuating limitations associated with ME/CFS, generating ongoing stress, uncertainty, and fear of financial collapse if any element of their support structure failed.

> *"I currently work part-time and receive a supplementary sickness benefit. If either party suddenly decides that this arrangement is no longer acceptable—whether it's NIHDI suspending my benefits or my employer wanting to adjust my contract to match the hours I can work—it would be a financial disaster. This is a constant source of stress."*

Flexibility in **employment arrangements** such as combining part-time work with disability benefits, remote work options, or sustained medical part-time contracts was seen as essential but **largely unavailable**.

> *"One should have the right to combine work and disability benefits (since it is currently not progressive, but rather decreasing)."*

Participants also underlined a general **lack of understanding within workplace environments**, which hindered disclosure and accommodation. They described the need for structural and attitudinal changes to support sustainable employment for individuals with chronic illnesses.

> *"People act as if you can't have needs just because you're still working. Yet there are things they could do to make life easier, which would help you manage work better. Your personal context– being single and without support– is also not taken into account"*

The absence of tailored vocational guidance and employment support was another recurring concern. Some continued to work despite fears of worsening their health, in part to maintain a sense of purpose and financial security.

> *"I also really miss guidance regarding work. I choose to keep working because I want to continue contributing, but I am sometimes afraid of further harming my health. Any form of support in finding better-adapted work would also be an enormous help."*

Similarly, household and gardening support, although clearly needed, remained inaccessible due to cost. Many participants described significant financial barriers, particularly for those over 60 who lose eligibility for certain reimbursements. Participants highlighted the disproportionate burden faced by those with **family responsibilities**. As one respondent noted, losing employment while caring for young children led to severe financial strain and emotional distress.

6.4.2.3 Psychosocial impact

Respondents articulated a **broad spectrum of negative emotions**, including frustration, guilt, anxiety, humiliation, loneliness, and financial strain. Many linked these feelings to societal stigma and repeated experiences of invalidation.

> *"Besides the frustrations and suffering that the illness already brings, the ignorance of others is very humiliating. Sometimes I don't even know what to say when people ask me what I do in life, and I try to avoid the question."*

The **availability of partners and informal caregivers** emerged as a key determinant of psychosocial burden. Participants emphasised that care needs, financial insecurity, and emotional stability were heavily contingent on the presence of supportive relationships. Families, especially parents and partners shouldering heavy emotional and financial responsibilities. The respondents acknowledged the profound impact of the illness on their loved ones, balancing gratitude with guilt and concern for the long-term sustainability of informal care.

> *"The impact on my partner and children cannot be underestimated. The careful way they deal with me – sometimes out of necessity – is both healing and valuable, but it doesn't make you feel strong."*

> *"The questionnaire does not take into account whether you have a partner who helps with care, finances, etc. My answers would be different if I didn't have a partner."*

Loss of identity emerged as a salient subtheme. Participants described profound struggles reconciling former roles and capacities with their current limitations. This identity disruption was accompanied by feelings of worthlessness and internalised stigma.

> *"I still struggle a lot with my own identity—who I even am anymore, what I still mean. I used to be such a busy, active woman with a career, and now I feel like I'm nothing. I'm in therapy for this, but on a reasonably good day I can lift myself up and accept 'it,' whereas on difficult days like today, my head drops again… and I torment myself with those questions, the 'why.' And then the guilt becomes very heavy, as well as the deep sense of shame."*

Respondents described substantial **social isolation**, arising not only from physical constraints but also from the lack of recognition or accommodation by others. Adjustments such as cancelling activities or avoiding overstimulating environments were frequently misunderstood, contributing to withdrawal and the progressive erosion of social networks. *first CFS, then despair – not the other way around.*

> *"Also social isolation not only relates to being homebound, but also to the fact that there is little understanding of the disease."*

Participants described the cumulative effect of physical decline, social isolation, and lack of medical recognition as prompting **existential distress**. This included explicit considerations of **end-of-life options** framed as responses to unbearable circumstances rather than as psychiatric depression.

> *"I still need medical care. I have to hold on and fight to continue receiving palliative treatment, since no curative treatment exists yet."*

Shrinking social circles, loss of informal support, and reduced participation in community life heightened feelings of uselessness and social impoverishment, particularly among those unable to work or engage meaningfully in society.

6.5 Limitations and conclusion

The open-ended question was included in the NEED survey to give participants the opportunity to express needs beyond those explicitly addressed in the survey. Following the qualitative analysis we found that participants reiterated needs that were already covered in the survey. This may be explained by the fact that participants tend to prioritise certain needs and emphasise those they experience most strongly. In addition, participants may use open-ended responses not only to identify needs, but also to elaborate on them for example by expressing how these needs affect them emotionally or in daily life, or by providing concrete examples. Notably, the most frequently expressed need was recognition, which had not been explicitly addressed in the survey. Recognition spans multiple levels, including society, policy, and healthcare providers.

Indeed, the lived experience of people with ME/CFS is fundamentally shaped by the degree to which their condition is recognised and understood across policy, healthcare and society. Insufficient clinical expertise, fragmented care structures, and persistent psychological misattribution form key contributing factors that undermine appropriate diagnosis, treatment, and disability evaluation. These shortcomings generate substantial consequences at the healthcare level, including inaccessible services, limited treatment practices, poor communication, and a lack of coordinated follow-up.

The figure demonstrates that these healthcare gaps extend far beyond the clinical setting, affecting nearly all domains of patients' daily lives. Unmet needs accumulate at the biopsychosocial level, influencing activities of daily living, quality of life, social participation, employment, financial security, psychosocial well-being, and personal identity. Participants' accounts show that the burden of the illness is compounded by societal misunderstanding, inadequate support systems, and the necessity of navigating care independently.

The participants underscored that improving recognition of ME/CFS i.e. grounded in updated scientific knowledge, evidence-based care, and validated diagnostic criteria is essential for reducing stigma and ensuring equitable access to appropriate care. Meaningful progress requires coordinated action across healthcare, social services, and policy structures, with patient experiences positioned at the center of care development.

7 SEARCH STRATEGY

7.1 Ovid Medline

Ovid MEDLINE(R) and Epub Ahead of Print, In-Process, In-Data-Review & Other Non-Indexed Citations, Daily and Versions <1946 to October 16, 2024		
Date: 16 October 2024		
1	Fatigue Syndrome, Chronic/	6421
2	(chronic* adj3 fatigue).ab,ti,kf.	10395
3	(fatigue* adj2 (disorder* or chronic or syndrome or persistent or severe or severity)).ti,ab,kf.	15368
4	(fatigue* adj2 (postviral or post viral or post infection* or postinfection* or immune dysfunction*)).ti,ab,kf.	246
5	((myalgic or post infection* or postinfection*) adj3 (encephalomyelitis or encephalopath*)).ti,ab,kf.	1908
6	cfs.ab,ti,kf.	10406
7	((ME adj CFS) or (CFS adj ME) or CFIDS or PVFS).ti,ab,kf.	1474
8	((CFS adj SEID) or (SEID adj CFS) or (ME adj CFS adj SEID) or (ME adj SEID) or (SEID adj ME)).ti,ab,kf.	10
9	((chronic adj2 epstein Barr virus) or CEBV or CAEBV or chronic mononucleosis).ti,ab,kf.	748
10	((Orthostatic intolerance or postural orthostatic tachycardia syndrome or postural tachycardia syndrome or POTS) adj6 (CFS or chronic* fatigue* or ME or myalgic or SEID or systemic exertion)).ti,ab,kf.	145
11	(Systemic Exertion Intolerance Disease or SEID).ti,ab,kf.	71
12	((Post-exertional or postexertional) adj2 malaise).ti,ab,kf.	391
13	(neurasthenic neuroses or epidemic neuromyasthenia or neurataxia or neuroasthenia or neurasthenia).ti,ab,kf.	965
14	effort syndrome*.ti,ab,kf.	89
15	2 or 3 or 4 or 5 or 6 or 7 or 8 or 9 or 10 or 11 or 12 or 13 or 14	24645
16	1 or 15	25346
17	"Delivery of Health Care"/	123835
18	"Delivery of Health Care, Integrated"/	14785
19	access to primary care/	45

20	patient care team/	70824
21	health services accessibility/	89817
22	Patient Care Management/	4750
23	Patient-Centered Care/	24102
24	exp Patient Care Planning/	69253
25	Comprehensive Health Care/	6865
26	algorithms/	320117
27	Critical Pathways/	8139
28	triage/	15718
29	17 or 18 or 19 or 20 or 21 or 22 or 23 or 24 or 25 or 26 or 27 or 28	707646
30	Access to Care.ab,ti,kf.	20176
31	Patient Management.ab,ti,kf.	26269
32	Care Coordination.ab,ti,kf.	6359
33	Health Care Delivery.ab,ti,kf.	13080
34	Multidisciplinary Care.ab,ti,kf.	5986
35	Continuity of Care.ab,ti,kf.	10294
36	Chronic Disease Management.ab,ti,kf.	3628
37	og.xs.	1086435
38	triage.ab,ti,kw,kf.	27505
39	stratified care.ab,ti,kw,kf.	207
40	algorithm?.ab,ti,kw,kf.	402272
41	"process of care".ab,ti,kw,kf.	2990
42	case management plan?.ab,ti,kw,kf.	34
43	care map?.ab,ti,kw,kf.	166
44	((clinical or critical or care or healthcare or health or patient or referral or tailored) adj2 (path or pathway)).ab,ti,kf.	16992
45	(health adj3 (centre? or structure or facility or facilities)).ab,ti,kf.	57607
46	((assessment or referral or specialized por specialist or outpatient?) adj2 (centre? or clinic? or team?)).ab,ti,kf.	61214
47	(structure adj2 (care or management)).ab,ti,kf.	1848
48	individual management.ab,ti,kf.	409
49	(multidimensional adj2 (management or therapy or care)).ab,ti,kf.	425
50	(multidisciplinary adj2 (team? or intervention? or care)).ab,ti,kf.	41624
51	(tailored adj2 (care or therapy or rehabilitation)).ab,ti,kf.	4477
52	primary care.ab,ti,kf.	156831
53	first-line care.ab,ti,kf.	93
54	((recovery or comprehensive) adj2 program).ab,ti,kf.	4762
55	((rehabilitation or management or assessment) adj2 service).ab,ti,kf.	6209
56	level of care.ab,ti,kf.	7728

57	30 or 31 or 32 or 33 or 34 or 35 or 36 or 37 or 38 or 39 or 40 or 41 or 42 or 43 or 44 or 45 or 46 or 47 or 48 or 49 or 50 or 51 or 52 or 53 or 54 or 55 or 56	1850414
58	29 or 57	2220720
59	16 and 58	1518
60	limit 59 to yr="2014-2024"	877
61	((systematic review or systematic literature review or systematic scoping review or systematic narrative review or systematic qualitative review or systematic evidence review or systematic quantitative review or systematic meta-review or systematic critical review or systematic mixed studies review or systematic mapping review or systematic cochrane review or "systematic search and review" or systematic integrative review).ti. not comment.pt. not (protocol or protocols).ti. not medline.st.) or (Cochrane Database Syst Rev.ja. and review.pt.) or systematic review.pt.	327926
62	meta-analys*.ti.	214094
63	meta-analysis.pt.	209995
64	exp Technology Assessment, Biomedical/	12486
65	health technology assessment.ti,kf.	3152
66	HtA.ti,kf.	959
67	61 or 62 or 63 or 64 or 65 or 66	460009
68	60 and 67	29
69	exp clinical pathway/	8139
70	exp clinical protocol/	196754
71	exp consensus/	23853
72	exp consensus development conference/	12859
73	exp consensus development conferences as topic/	3012
74	critical pathways/	8139
75	exp guideline/	39481
76	guidelines as topic/	42350
77	exp practice guideline/	32439
78	practice guidelines as topic/	130234
79	health planning guidelines/	4165
80	(guideline or practice guideline or consensus development conference or consensus development conference, NIH).pt.	49516
81	(position statement* or policy statement* or practice parameter* or best practice*).ti,ab,kf,kw.	52357
82	(standards or guideline or guidelines).ti,kf,kw.	146715
83	((practice or treatment* or clinical) adj guideline*).ab.	59085
84	(CPG or CPGs).ti.	6671
85	consensus*.ti,kf,kw.	39151
86	consensus*.ab. /freq=2	38719
87	((critical or clinical or practice) adj2 (path or paths or pathway or pathways or protocol*)).ti,ab,kf,kw.	29748
88	recommendat*.ti,kf,kw.	58890

89	(care adj2 (standard or path or paths or pathway or pathways or map or maps or plan or plans)).ti,ab,kf,kw.	95513
90	(algorithm* adj2 (screening or examination or test or tested or testing or assessment* or diagnosis or diagnoses or diagnosed or diagnosing)).ti,ab,kf,kw.	11489
91	(algorithm* adj2 (pharmacotherap* or chemotherap* or chemotreatment* or therap* or treatment* or intervention*)).ti,ab,kf,kw.	14236
92	or/69-91	800168
93	60 and 92	98
94	68 or 93	123
95	60 not (68 or 93)	754

Ovid MEDLINE(R) and Epub Ahead of Print, In-Process, In-Data-Review & Other Non-Indexed Citations, Daily and Versions <1946 to October 16, 2024		
Date: 06 March 2026		
1	Fatigue Syndrome, Chronic/	6669
2	(chronic* adj3 fatigue).ab,ti,kf.	11319
3	(fatigue* adj2 (disorder* or chronic or syndrome or persistent or severe or severity)).ti,ab,kf.	17715
4	(fatigue* adj2 (postviral or post viral or post infection* or postinfection* or immune dysfunction*)).ti,ab,kf.	266
5	((myalgic or post infection* or postinfection*) adj3 (encephalomyelitis or encephalopath*)).ti,ab,kf.	2261
6	cfs.ab,ti,kf.	11987
7	((ME adj CFS) or (CFS adj ME) or CFIDS or PVFS).ti,ab,kf.	1801
8	((CFS adj SEID) or (SEID adj CFS) or (ME adj CFS adj SEID) or (ME adj SEID) or (SEID adj ME)).ti,ab,kf.	10
9	((chronic adj2 epstein Barr virus) or CEBV or CAEBV or chronic mononucleosis).ti,ab,kf.	811
10	((Orthostatic intolerance or postural orthostatic tachycardia syndrome or postural tachycardia syndrome or POTS) adj6 (CFS or chronic* fatigue* or ME or myalgic or SEID or systemic exertion)).ti,ab,kf.	173
11	(Systemic Exertion Intolerance Disease or SEID).ti,ab,kf.	75
12	((Post-exertional or postexertional) adj2 malaise).ti,ab,kf.	536
13	(neurasthenic neuroses or epidemic neuromyasthenia or neurataxia or neuroasthenia or neurasthenia).ti,ab,kf.	981
14	effort syndrome*.ti,ab,kf.	89
15	2 or 3 or 4 or 5 or 6 or 7 or 8 or 9 or 10 or 11 or 12 or 13 or 14	27893
16	1 or 15	28594
17	"Delivery of Health Care"/	126960
18	"Delivery of Health Care, Integrated"/	15603
19	access to primary care/	63
20	patient care team/	73175
21	health services accessibility/	97670

22	Patient Care Management/	4762
23	Patient-Centered Care/	25949
24	exp Patient Care Planning/	70832
25	Comprehensive Health Care/	6948
26	algorithms/	347320
27	Critical Pathways/	8547
28	triage/	16754
29	17 or 18 or 19 or 20 or 21 or 22 or 23 or 24 or 25 or 26 or 27 or 28	752293
30	Access to Care.ab,ti,kf.	23700
31	Patient Management.ab,ti,kf.	30218
32	Care Coordination.ab,ti,kf.	7524
33	Health Care Delivery.ab,ti,kf.	13923
34	Multidisciplinary Care.ab,ti,kf.	8667
35	Continuity of Care.ab,ti,kf.	11939
36	Chronic Disease Management.ab,ti,kf.	4633
37	og.xs.	1125927
38	triage.ab,ti,kw,kf.	31464
39	stratified care.ab,ti,kw,kf.	252
40	algorithm?.ab,ti,kw,kf.	463455
41	"process of care".ab,ti,kw,kf.	3086
42	case management plan?.ab,ti,kw,kf.	37
43	care map?.ab,ti,kw,kf.	182
44	((clinical or critical or care or healthcare or health or patient or referral or tailored) adj2 (path or pathway)).ab,ti,kf.	19925
45	(health adj3 (centre? or structure or facility or facilities)).ab,ti,kf.	63162
46	((assessment or referral or specialized por specialist or outpatient?) adj2 (centre? or clinic? or team?)).ab,ti,kf.	66554
47	(structure adj2 (care or management)).ab,ti,kf.	1987
48	individual management.ab,ti,kf.	445
49	(multidimensional adj2 (management or therapy or care)).ab,ti,kf.	535
50	(multidisciplinary adj2 (team? or intervention? or care)).ab,ti,kf.	50419
51	(tailored adj2 (care or therapy or rehabilitation)).ab,ti,kf.	6099
52	primary care.ab,ti,kf.	171437
53	first-line care.ab,ti,kf.	114
54	((recovery or comprehensive) adj2 program).ab,ti,kf.	5204
55	((rehabilitation or management or assessment) adj2 service).ab,ti,kf.	6799
56	level of care.ab,ti,kf.	8548
57	30 or 31 or 32 or 33 or 34 or 35 or 36 or 37 or 38 or 39 or 40 or 41 or 42 or 43 or 44 or 45 or 46 or 47 or 48 or 49 or 50 or 51 or 52 or 53 or 54 or 55 or 56	1996099
58	29 or 57	2384955

59	16 and 58	1736
60	limit 59 to yr="2014-2024"	894
61	((systematic review or systematic literature review or systematic scoping review or systematic narrative review or systematic qualitative review or systematic evidence review or systematic quantitative review or systematic meta-review or systematic critical review or systematic mixed studies review or systematic mapping review or systematic cochrane review or "systematic search and review" or systematic integrative review).ti. not comment.pt. not (protocol or protocols).ti. not medline.st.) or (Cochrane Database Syst Rev.ja. and review.pt.) or systematic review.pt.	391544
62	meta-analys*.ti.	257031
63	meta-analysis.pt.	225747
64	exp Technology Assessment, Biomedical/	13004
65	health technology assessment.ti,kf.	3645
66	HtA.ti,kf.	1152
67	61 or 62 or 63 or 64 or 65 or 66	531899
68	60 and 67	28
69	exp clinical pathway/	8547
70	exp clinical protocol/	205338
71	exp consensus/	24905
72	exp consensus development conference/	21621
73	exp consensus development conferences as topic/	4306
74	critical pathways/	8547
75	exp guideline/	42742
76	guidelines as topic/	42747
77	exp practice guideline/	36063
78	practice guidelines as topic/	134361
79	health planning guidelines/	4170
80	(guideline or practice guideline or consensus development conference or consensus development conference, NIH).pt.	61097
81	(position statement* or policy statement* or practice parameter* or best practice*).ti,ab,kf,kw.	59598
82	(standards or guideline or guidelines).ti,kf,kw.	160303
83	((practice or treatment* or clinical) adj guideline*).ab.	67382
84	(CPG or CPGs).ti.	6926
85	consensus*.ti,kf,kw.	44598
86	consensus*.ab. /freq=2	44670
87	((critical or clinical or practice) adj2 (path or paths or pathway or pathways or protocol*)).ti,ab,kf,kw.	35843
88	recommendat*.ti,kf,kw.	65209
89	(care adj2 (standard or path or paths or pathway or pathways or map or maps or plan or plans)).ti,ab,kf,kw.	110741

90	(algorithm* adj2 (screening or examination or test or tested or testing or assessment* or diagnosis or diagnoses or diagnosed or diagnosing)).ti,ab,kf,kw.	12694
91	(algorithm* adj2 (pharmacotherap* or chemotherap* or chemotreatment* or therap* or treatment* or intervention*)).ti,ab,kf,kw.	15814
92	or/69-91	871520
93	60 and 92	102
94	68 or 93	126
95	60 not (68 or 93)	768
96	limit 68 to dt="20241016-20260309"	0
97	limit 93 to dt="20241016-20260309"	3
98	limit 95 to dt="20241016-20260309"	25

7.2 Cochrane

Cochrane		
Date: 24 October 2024		
#1	[mh ^"Fatigue Syndrome, Chronic"]	568
#2	(chronic* NEAR/3 fatigue):ab,ti,kw	2800
#3	(fatigue* NEAR/2 (disorder* or chronic or syndrome or persistent or severe or severity)):ab,ti,kw	4658
#4	(fatigue* NEAR/2 (postviral or "post viral" or (post NEXT/1 infection*) or postinfection* or (immune NEXT/1 dysfunction*))):ab,ti,kw	37
#5	((myalgic or (post NEXT/1 infection*) or postinfection*) NEAR/3 (encephalomyelitis or encephalopath*)):ab,ti,kw	146
#6	cfs:ab,ti,kw	1196
#7	((ME NEAR/1 CFS) or (CFS NEAR/1 ME) or CFIDS or PVFS):ab,ti,kw	144
#8	((CFS NEAR/1 SEID) or (SEID NEAR/1 CFS) or (ME NEAR/1 CFS NEAR/1 SEID) or (ME NEAR/1 SEID) or (SEID NEAR/1 ME)):ab,ti,kw	0
#9	((chronic NEAR/2 "epstein Barr virus") or CEBV or CAEBV or "chronic mononucleosis"):ab,ti,kw	12
#10	(("Orthostatic intolerance" or "postural orthostatic tachycardia syndrome" or "postural tachycardia syndrome" or POTS) NEAR/6 (CFS or (chronic* NEXT/1 fatigue*) or ME or myalgic or SEID or "systemic exertion")):ab,ti,kw	9
#11	("Systemic Exertion Intolerance Disease" or SEID):ab,ti,kw	14
#12	((Post-exertional or postexertional) NEAR/2 malaise):ab,ti,kw	64
#13	("neurasthenic neuroses" or "epidemic neuromyasthenia" or neurataxia or neuroasthenia or neurasthenia):ab,ti,kw	97
#14	(effort next/1 syndrome*):ab,ti,kw	1
#15	#2 or #3 or #4 or #5 or #6 or #7 or #8 or #9 or #10 or #11 or #12 or #13 or #14	6436
#16	#1 or #15	6436
#17	[mh ^"Delivery of Health Care"]	1630
#18	[mh ^"Delivery of Health Care, Integrated"]	608
#19	[mh ^"access to primary care"]	0
#20	[mh ^"patient care team"]	2284

#21	[mh ^"health services accessibility"]	1130
#22	[mh ^"Patient Care Management"]	195
#23	[mh ^"Patient-Centered Care"]	1022
#24	[mh "Patient Care Planning"]	2536
#25	[mh ^"Comprehensive Health Care"]	101
#26	[mh ^"algorithms"]	5240
#27	[mh ^"Critical Pathways"]	328
#28	[mh ^"triage"]	483
#29	#17 or #18 or #19 or #20 or #21 or #22 or #23 or #24 or #25 or #26 or #27 or #28	14295
#30	"Access to Care":ab,ti,kw	921
#31	"Patient Management":ab,ti,kw	1466
#32	"Care Coordination":ab,ti,kw	768
#33	"Health Care Delivery":ab,ti,kw	2467
#34	"Multidisciplinary Care":ab,ti,kw	570
#35	"Continuity of Care":ab,ti,kw	714
#36	"Chronic Disease Management":ab,ti,kw	549
#37	[mh /OG]	9749
#38	triage:ab,ti,kw	2219
#39	"stratified care":ab,ti,kw	79
#40	algorithm?:ab,ti,kw	19253
#41	"process of care":ab,ti,kw	349
#42	("case management" NEXT/1 plan?):ab,ti,kw	2
#43	(care NEXT/1 map?):ab,ti,kw	18
#44	((clinical or critical or care or healthcare or health or patient or referral or tailored) NEAR/2 (path or pathway)):ab,ti,kw	1974
#45	(health NEAR/3 (centre? or structure or facility or facilities)):ab,ti,kw	11109
#46	((assessment or referral or specialized or specialist or outpatient?) NEAR/2 (centre? or clinic? or team?)):ab,ti,kw	15749
#47	(structure NEAR/2 (care or management)):ab,ti,kw	79
#48	"individual management":ab,ti,kw	40
#49	(multidimensional NEAR/2 (management or therapy or care)):ab,ti,kw	123
#50	(multidisciplinary NEAR/2 (team? or intervention? or care)):ab,ti,kw	4003
#51	(tailored NEAR/2 (care or therapy or rehabilitation)):ab,ti,kw	884
#52	"primary care":ab,ti,kw	23640
#53	"first-line care":ab,ti,kw	29
#54	((recovery or comprehensive) NEAR/2 program):ab,ti,kw	1387
#55	((rehabilitation or management or assessment) NEAR/2 service):ab,ti,kw	1008
#56	"level of care":ab,ti,kw	475

#57	#30 or #31 or #32 or #33 or #34 or #35 or #36 or #37 or #38 or #39 or #40 or #41 or #42 or #43 or #44 or #45 or #46 or #47 or #48 or #49 or #50 or #51 or #52 or #53 or #54 or #55 or #56	88399
#58	#29 or #57	92766
#59	#16 and #58	367
#60	#59 with Cochrane Library publication date Between Jan 2014 and Dec 2024, in Cochrane Reviews	2
#61	#59 with Publication Year from 2014 to 2024, in Trials	254
#62	#14 and #60	0

Cochrane		
Date: 10 March 2026		
#1	[mh ^"Fatigue Syndrome, Chronic"]	574
#2	(chronic* NEAR/3 fatigue):ab,ti,kw	3092
#3	(fatigue* NEAR/2 (disorder* or chronic or syndrome or persistent or severe or severity)):ab,ti,kw	v
#4	(fatigue* NEAR/2 (postviral or "post viral" or (post NEXT/1 infection*) or postinfection* or (immune NEXT/1 dysfunction*))):ab,ti,kw	46
#5	((myalgic or (post NEXT/1 infection*) or postinfection*) NEAR/3 (encephalomyelitis or encephalopath*)):ab,ti,kw	175
#6	cfs:ab,ti,kw	1394
#7	((ME NEAR/1 CFS) or (CFS NEAR/1 ME) or CFIDS or PVFS):ab,ti,kw	170
#8	((CFS NEAR/1 SEID) or (SEID NEAR/1 CFS) or (ME NEAR/1 CFS NEAR/1 SEID) or (ME NEAR/1 SEID) or (SEID NEAR/1 ME)):ab,ti,kw	0
#9	((chronic NEAR/2 "epstein Barr virus") or CEBV or CAEBV or "chronic mononucleosis"):ab,ti,kw	13
#10	(("Orthostatic intolerance" or "postural orthostatic tachycardia syndrome" or "postural tachycardia syndrome" or POTS) NEAR/6 (CFS or (chronic* NEXT/1 fatigue*) or ME or myalgic or SEID or "systemic exertion")):ab,ti,kw	9
#11	("Systemic Exertion Intolerance Disease" or SEID):ab,ti,kw	18
#12	((Post-exertional or postexertional) NEAR/2 malaise):ab,ti,kw	98
#13	("neurasthenic neuroses" or "epidemic neuromyasthenia" or neurataxia or neuroasthenia or neurasthenia):ab,ti,kw	100
#14	(effort next/1 syndrome*):ab,ti,kw	1
#15	#2 or #3 or #4 or #5 or #6 or #7 or #8 or #9 or #10 or #11 or #12 or #13 or #14	7380
#16	#1 or #15	7380
#17	[mh ^"Delivery of Health Care"]	1602
#18	[mh ^"Delivery of Health Care, Integrated"]	633
#19	[mh ^"access to primary care"]	0
#20	[mh ^"patient care team"]	2322
#21	[mh ^"health services accessibility"]	1187
#22	[mh ^"Patient Care Management"]	193
#23	[mh ^"Patient-Centered Care"]	1076

#24	[mh "Patient Care Planning"]	2564
#25	[mh ^"Comprehensive Health Care"]	100
#26	[mh ^"algorithms"]	5270
#27	[mh ^"Critical Pathways"]	331
#28	[mh ^"triage"]	514
#29	#17 or #18 or #19 or #20 or #21 or #22 or #23 or #24 or #25 or #26 or #27 or #28	14528
#30	"Access to Care":ab,ti,kw	1032
#31	"Patient Management":ab,ti,kw	1606
#32	"Care Coordination":ab,ti,kw	855
#33	"Health Care Delivery":ab,ti,kw	2503
#34	"Multidisciplinary Care":ab,ti,kw	66
#35	"Continuity of Care":ab,ti,kw	796
#36	"Chronic Disease Management":ab,ti,kw	643
#37	[mh /OG]	10112
#38	triage:ab,ti,kw	2406
#39	"stratified care":ab,ti,kw	87
#40	algorithm?:ab,ti,kw	20577
#41	"process of care":ab,ti,kw	351
#42	("case management" NEXT/1 plan?):ab,ti,kw	3
#43	(care NEXT/1 map?):ab,ti,kw	18
#44	((clinical or critical or care or healthcare or health or patient or referral or tailored) NEAR/2 (path or pathway)):ab,ti,kw	2194
#45	(health NEAR/3 (centre? or structure or facility or facilities)):ab,ti,kw	11873
#46	((assessment or referral or specialized or specialist or outpatient?) NEAR/2 (centre? or clinic? or team?)):ab,ti,kw	16713
#47	(structure NEAR/2 (care or management)):ab,ti,kw	76
#48	"individual management":ab,ti,kw	44
#49	(multidimensional NEAR/2 (management or therapy or care)):ab,ti,kw	137
#50	(multidisciplinary NEAR/2 (team? or intervention? or care)):ab,ti,kw	4472
#51	(tailored NEAR/2 (care or therapy or rehabilitation)):ab,ti,kw	1041
#52	"primary care":ab,ti,kw	24864
#53	"first-line care":ab,ti,kw	32
#54	((recovery or comprehensive) NEAR/2 program):ab,ti,kw	1524
#55	((rehabilitation or management or assessment) NEAR/2 service):ab,ti,kw	1026
#56	"level of care":ab,ti,kw	496
#57	#30 or #31 or #32 or #33 or #34 or #35 or #36 or #37 or #38 or #39 or #40 or #41 or #42 or #43 or #44 or #45 or #46 or #47 or #48 or #49 or #50 or #51 or #52 or #53 or #54 or #55 or #56	94320
#58	#29 or #57	98784
#59	#16 and #58	424

#60	#59 with Cochrane Library publication date Between Oct 2024 and Mar 2026, in Cochrane Reviews	0
#61	#59 with Cochrane Library publication date Between Oct 2024 and Mar 2026, in Trials	68

7.3 Cinahl

Cinahl		
Date: 26 November 2024		
S1	MH Fatigue Syndrome, Chronic	3,451
S2	AB (chronic* N3 fatigue) OR TI (chronic* N3 fatigue)	4,250
S3	AB (fatigue* N2 (disorder* or chronic or syndrome or persistent or severe or severity)) OR TI (fatigue* N2 (disorder* or chronic or syndrome or persistent or severe or severity))	6,796
S4	AB (fatigue* N2 (postviral or post viral or post infection* or postinfection* or immune dysfunction*)) OR TI (fatigue* N2 (postviral or post viral or post infection* or postinfection* or immune dysfunction*))	69
S5	AB ((myalgic or post infection* or postinfection*) N3 (encephalomyelitis or encephalopath*)) OR TI ((myalgic or post infection* or postinfection*) N3 (encephalomyelitis or encephalopath*))	658
S6	AB cfs OR TI cfs	2,418
S7	AB ((ME N1 CFS) or (CFS N1 ME) or CFIDS or PVFS) OR TI ((ME N1 CFS) or (CFS N1 ME) or CFIDS or PVFS)	598
S8	AB ((CFS N1 SEID) or (SEID N1 CFS) or (ME N1 CFS N1 SEID) or (ME N1 SEID) or (SEID N1 ME)) OR TI ((CFS N1 SEID) or (SEID N1 CFS) or (ME N1 CFS N1 SEID) or (ME N1 SEID) or (SEID N1 ME))	2
S9	AB ((chronic N2 epstein Barr virus) or CEBV or CAEBV or chronic mononucleosis) OR TI ((chronic N2 epstein Barr virus) or CEBV or CAEBV or chronic mononucleosis)	127
S10	AB ((Orthostatic intolerance or postural orthostatic tachycardia syndrome or postural tachycardia syndrome or POTS) N6 (CFS or chronic* fatigue* or ME or myalgic or SEID or systemic exertion)) OR TI ((Orthostatic intolerance or postural orthostatic tachycardia syndrome or postural tachycardia syndrome or POTS) N6 (CFS or chronic* fatigue* or ME or myalgic or SEID or systemic exertion))	42
S11	AB (Systemic Exertion Intolerance Disease or SEID) OR TI (Systemic Exertion Intolerance Disease or SEID)	28
S12	AB ((Post-exertional or postexertional) N2 malaise) OR TI ((Post-exertional or postexertional) N2 malaise)	104
S13	AB (neurasthenic neuroses or epidemic neuromyasthenia or neurataxia or neuroasthenia or neurasthenia) OR TI (neurasthenic neuroses or epidemic neuromyasthenia or neurataxia or neuroasthenia or neurasthenia)	95
S14	AB effort syndrome* OR TI effort syndrome*	180
S15	(S2 OR S3 OR S4 OR S5 OR S6 OR S7 OR S8 OR S9 OR S10 OR S11 OR S12 OR S13 OR S14)	8,622
S16	S1 OR S15	9,340
S17	MH Health Care Delivery	69,412
S18	MH Health Care Delivery, Integrated	0
S19	MH Access to Primary Care	235
S20	MH Multidisciplinary Care Team	54,253

S21	MH Patient Care Plans	7,797
S22	MH Health Services Accessibility	115,300
S23	MH Managed Care Programs	12,012
S24	MH Patient Centered Care	37,906
S25	MH Advance Care Planning	5,353
S26	MH Algorithms	47,411
S27	MH Critical Path	6,214
S28	MH Triage	11,464
S29	S17 or S18 or S19 or S20 or S21 or S22 or S23 or S24 or S25 or S26 or S27 or S28	348,913
S30	AB Access to Care OR TI Access to Care	31,556
S31	AB Patient Management OR TI Patient Management	83,042
S32	AB Care Coordination OR TI Care Coordination	7,462
S33	AB Health Care Delivery OR TI Health Care Delivery	14,710
S34	AB Multidisciplinary Care OR TI Multidisciplinary Care	8,128
S35	AB Continuity of Care OR TI Continuity of Care	8,947
S36	AB Chronic Disease Management OR TI Chronic Disease Management	6,078
S37	AB triage OR TI triage	13,040
S38	AB stratified care OR TI stratified care	909
S39	AB algorithm? OR TI algorithm?	49,941
S40	AB "process of care" OR TI "process of care"	1,669
S41	AB case management plan? OR TI case management plan?	186
S42	AB care map? OR TI care map?	624
S43	AB ((clinical or critical or care or healthcare or health or patient or referral or tailored) N2 (path or pathway)) OR TI ((clinical or critical or care or healthcare or health or patient or referral or tailored) N2 (path or pathway))	17,367
S44	AB (health N3 (centre? or structure or facility or facilities)) OR TI (health N3 (centre? or structure or facility or facilities))	44,705
S45	AB ((assessment or referral or specialized por specialist or outpatient?) N2 (centre? or clinic? or team?)) OR TI ((assessment or referral or specialized por specialist or outpatient?) N2 (centre? or clinic? or team?))	32,130
S46	AB (structure N2 (care or management)) OR TI (structure N2 (care or management))	2,307
S47	AB individual management OR TI individual management	4,775
S48	AB (multidimensional N2 (management or therapy or care)) OR TI (multidimensional N2 (management or therapy or care))	471
S49	AB (multidisciplinary N2 (team? or intervention? or care)) OR TI (multidisciplinary N2 (team? or intervention? or care))	19,444
S50	AB (tailored N2 (care or therapy or rehabilitation)) OR TI (tailored N2 (care or therapy or rehabilitation))	2,366
S51	AB primary care OR TI primary care	107,613
S52	AB first-line care OR TI first-line care	624
S53	AB ((recovery or comprehensive) N2 program) OR TI ((recovery or comprehensive) N2 program)	6,134

S54	AB ((rehabilitation or management or assessment) N2 service) OR TI ((rehabilitation or management or assessment) N2 service)	12,601
S55	AB level of care OR TI level of care	41,840
S56	S30 or S31 or S32 or S33 or S34 or S35 or S36 or S37 or S38 or S39 or S40 or S41 or S42 or S43 or S44 or S45 or S46 or S47 or S48 or S49 or S50 or S51 or S52 or S53 or S54 or S55	438,325
S57	S29 or S56	690,560
S58	S16 and S57	988
S59	S58 AND PY 2014-2024	642
S60	S59 - Exclude MEDLINE records	402
S61	S60 - Peer Reviewed	387
S62	(TI (systematic* n3 review*)) or (AB (systematic* n3 review*)) or (TI (systematic* n3 bibliographic*)) or (AB (systematic* n3 bibliographic*)) or (TI (systematic* n3 literature)) or (AB (systematic* n3 literature)) or (TI (comprehensive* n3 literature)) or (AB (comprehensive* n3 literature)) or (TI (comprehensive* n3 bibliographic*)) or (AB (comprehensive* n3 bibliographic*)) or (TI (integrative n3 review)) or (AB (integrative n3 review)) or (JN “Cochrane Database of Systematic Reviews”) or (TI (information n2 synthesis)) or (TI (data n2 synthesis)) or (AB (information n2 synthesis)) or (AB (data n2 synthesis)) or (TI (data n2 extract*)) or (AB (data n2 extract*)) or (TI (medline or pubmed or psyclit or cinahl or (psycinfo not “psycinfo database”) or “web of science” or scopus or embase)) or (AB (medline or pubmed or psyclit or cinahl or (psycinfo not “psycinfo database”) or “web of science” or scopus or embase)) or (MH “Systematic Review”) or (MH “Meta Analysis”) or (TI (meta-analy* or metaanaly*)) or (AB (meta-analy* or metaanaly*))	318,849
S63	S61 AND S62	27
S64	PT (practice guidelines or standards or protocol or critical path or care plan) or TI (guideline* or standards or consensus* or recommendat*) or AU (guideline* or standards or consensus* or recommendat*) or CA (guideline* or standards or consensus* or recommendat*) or TI ("practice parameter*" or "position statement*" or "policy statement*" or CPG or CPGs or "best practice*") or TI (care N2 path or care N2 paths or care N2 pathway or care N2 pathways or care N2 map or care N2 maps or care N2 plan or care N2 plans or care N2 standard*) or TI (critical N2 path or critical N2 paths or critical N2 pathway or critical N2 pathways or critical N2 protocol* or clinical N2 path or clinical N2 paths or clinical N2 pathway or clinical N2 pathways or clinical N2 protocol* or practice N2 path or practice N2 paths or practice N2 pathway or practice N2 pathways or practice N2 protocol*) or TI (algorithm* AND (pharmacotherap* or chemotherap* or chemotreatment* or therap* or treatment* or intervention*)) or (PT algorithm AND TI (pharmacotherap* or chemotherap* or chemotreatment* or therap* or treatment* or intervention*)) or TI (algorithm* AND (screening or examination or test or tested or testing or assessment* or diagnosis or diagnoses or diagnosed or diagnosing)) or (PT algorithm AND TI (screening or examination or test or tested or testing or assessment* or diagnosis or diagnoses or diagnosed or diagnosing))	174,061
S65	S61 AND S64	26
S66	S61 NOT(S63 OR S65)	335

Cinahl		
Date: 12 March 2026		
S1	((MH Fatigue Syndrome, Chronic) OR AB (chronic* N3 fatigue) OR TI (chronic* N3 fatigue) OR AB (fatigue* N2 (disorder* or chronic or syndrome or persistent or severe or severity)) OR TI (fatigue* N2 (disorder* or chronic or syndrome or persistent or severe or severity)) OR AB (fatigue* N2 (postviral or post viral or post infection* or postinfection* or immune dysfunction*)) OR TI (fatigue* N2 (postviral or post viral or post infection* or postinfection* or immune dysfunction*)) OR AB ((myalgic or post infection* or postinfection*) N3 (encephalomyelitis or encephalopath*)) OR TI ((myalgic or post infection* or postinfection*) N3 (encephalomyelitis or encephalopath*)) OR AB cfs OR TI cfs OR AB ((ME N1 CFS) or (CFS N1 ME) or CFIDS or PVFS) OR TI ((ME N1 CFS) or (CFS N1 ME) or CFIDS or PVFS) OR AB ((CFS N1 SEID) or (SEID N1 CFS) or (ME N1 CFS N1 SEID) or (ME N1 SEID) or (SEID N1 ME)) OR TI ((CFS N1 SEID) or (SEID N1 CFS) or (ME N1 CFS N1 SEID) or (ME N1 SEID) or (SEID N1 ME)) OR AB ((chronic N2 epstein Barr virus) or CEBV or CAEBV or chronic mononucleosis) OR TI ((chronic N2 epstein Barr virus) or CEBV or CAEBV or chronic mononucleosis) OR AB ((Orthostatic intolerance or postural orthostatic tachycardia syndrome or postural tachycardia syndrome or POTS) N6 (CFS or chronic* fatigue* or ME or myalgic or SEID or systemic exertion)) OR TI ((Orthostatic intolerance or postural orthostatic tachycardia syndrome or postural tachycardia syndrome or POTS) N6 (CFS or chronic* fatigue* or ME or myalgic or SEID or systemic exertion)) OR AB (Systemic Exertion Intolerance Disease or SEID) OR TI (Systemic Exertion Intolerance Disease or SEID) OR AB ((Post-exertional or postexertional) N2 malaise) OR TI ((Post-exertional or postexertional) N2 malaise) OR AB (neurasthenic neuroses or epidemic neuromyasthenia or neurataxia or neuroasthenia or neurasthenia) OR TI (neurasthenic neuroses or epidemic neuromyasthenia or neurataxia or neuroasthenia or neurasthenia) OR AB effort syndrome* OR TI effort syndrome*) AND ((MH Health Care Delivery) OR (MH Health Care Delivery, Integrated) OR (MH Access to Primary Care) OR (MH Multidisciplinary Care Team) OR (MH Patient Care Plans) OR (MH Health Services Accessibility) OR (MH Managed Care Programs) OR (MH Patient Centered Care) OR (MH Advance Care Planning) OR (MH Algorithms) OR (MH Critical Path) OR (MH Triage) OR AB Access to Care OR TI Access to Care OR AB Patient Management OR TI Patient Management OR AB Care Coordination OR TI Care Coordination OR AB Health Care Delivery OR TI Health Care Delivery OR AB Multidisciplinary Care OR TI Multidisciplinary Care OR AB Continuity of Care OR TI Continuity of Care OR AB Chronic Disease Management OR TI Chronic Disease Management OR AB triage OR TI triage OR AB stratified care OR TI stratified care OR AB algorithm? OR TI algorithm? OR AB "process of care" OR TI "process of care" OR AB case management plan? OR TI case management plan? OR AB care map? OR TI care map? OR AB ((clinical or critical or care or healthcare or health or patient or referral or tailored) N2 (path or pathway)) OR TI ((clinical or critical or care or healthcare or health or patient or referral or tailored) N2 (path or pathway)) OR AB (health N3 (centre? or structure or facility or facilities)) OR TI (health N3 (centre? or structure or facility or facilities)) OR AB ((assessment or referral or specialized por specialist or outpatient?) N2 (centre? or clinic? or team?)) OR TI ((assessment or referral or specialized por specialist or outpatient?) N2 (centre? or clinic? or team?)) OR AB (structure N2 (care or management)) OR TI (structure N2 (care or management)) OR AB individual management OR TI individual management OR AB (multidimensional N2 (management or therapy or care)) OR TI (multidimensional N2 (management or therapy or care)) OR AB (multidisciplinary N2 (team? or intervention? or care)) OR TI (multidisciplinary N2 (team? or intervention? or care)) OR AB (tailored N2 (care or therapy or rehabilitation)) OR TI (tailored N2 (care or therapy or rehabilitation)) OR AB primary care OR TI primary care OR AB first-line care OR TI first-line care OR AB ((recovery or comprehensive) N2 program) OR TI ((recovery or comprehensive) N2 program) OR AB ((rehabilitation or management or assessment) N2 service) OR TI ((rehabilitation or management or assessment) N2 service) OR AB level of care OR TI level of care) AND PY 2024-2026	127

7.4 Embase

Embase		
Date: 26 November 2024		
#1	'chronic fatigue syndrome'/exp	21975
#2	(chronic* NEAR/3 fatigue):ab,ti,kw	14665
#3	(fatigue* NEAR/2 (disorder* OR chronic OR syndrome OR persistent OR severe OR severity)):ab,ti,kw	23656
#4	(fatigue* NEAR/2 (postviral OR 'post viral' OR 'post infection*' OR postinfection* OR 'immune dysfunction*')):ab,ti,kw	303
#5	((myalgic OR 'post infection*' OR postinfection*) NEAR/3 (encephalomyelitis OR encephalopath*)):ab,ti,kw	2183
#6	cfs:ab,ti,kw	14137
#7	cfids:ab,ti,kw OR pvfs:ab,ti,kw	141
#8	(me NEAR/1 seid):ab,ti,kw	4
#9	((chronic NEAR/2 'epstein barr virus'):ab,ti,kw) OR cebv:ab,ti,kw OR caebv:ab,ti,kw OR 'chronic mononucleosis':ab,ti,kw	947
#10	(('orthostatic intolerance' OR 'postural orthostatic tachycardia syndrome' OR 'postural tachycardia syndrome' OR pots) NEAR/6 (cfs OR 'chronic* fatigue*' OR me OR myalgic OR seid OR 'systemic exertion')):ab,ti,kw	163
#11	'systemic exertion intolerance disease':ab,ti,kw OR seid:ab,ti,kw	118
#12	(('post-exertional' OR postexertional) NEAR/2 malaise):ab,ti,kw	485
#13	'neurasthenic neuroses':ab,ti,kw OR 'epidemic neuromyasthenia':ab,ti,kw OR neurataxia:ab,ti,kw OR neuroasthenia:ab,ti,kw OR neurasthenia:ab,ti,kw	961
#14	'effort syndrome*':ab,ti,kw	78
#15	#2 OR #3 OR #4 OR #5 OR #6 OR #7 OR #8 OR #9 OR #10 OR #11 OR #12 OR #13 OR #14	36689
#16	#1 OR #15	48253
#17	'health care delivery'/de	220937
#18	'integrated health care system'/de	14302
#19	'primary care access'/de	367
#20	'collaborative care team'/de	4351
#21	'health care access'/de	99956
#22	'patient care'/de	382686
#23	'person centered care'/de	2207
#24	'patient care planning'/exp	31356
#25	'health care'/de	140961
#26	'algorithm'/de	402694
#27	'clinical pathway'/de	10632
#28	'triage'/de	7538
#29	#17 OR #18 OR #19 OR #20 OR #21 OR #22 OR #23 OR #24 OR #25 OR #26 OR #27 OR #28	1228409
#30	'access to care':ab,ti,kw	24196
#31	'patient management':ab,ti,kw	40596

#32	'care coordination':ab,ti,kw	9313
#33	'health care delivery':ab,ti,kw	16135
#34	'multidisciplinary care':ab,ti,kw	9615
#35	'continuity of care':ab,ti,kw	13839
#36	'chronic disease management':ab,ti,kw	4776
#37	triage:ab,ti,kw	42642
#38	'stratified care':ab,ti,kw	283
#39	algorithm$:ab,ti,kw	510578
#40	'process of care':ab,ti,kw	3563
#41	'case management plan$':ab,ti,kw	45
#42	'care map$':ab,ti,kw	257
#43	((clinical OR critical OR care OR healthcare OR health OR patient OR referral OR tailored) NEAR/2 (path OR pathway)):ab,ti,kw	29125
#44	(health NEAR/3 (centre$ OR structure OR facility OR facilities)):ab,ti,kw	72024
#45	((assessment OR referral OR specialized OR specialist OR outpatient$) NEAR/2 (centre$ OR clinic$ OR team$)):ab,ti,kw	132897
#46	(structure NEAR/2 (care OR management)):ab,ti,kw	2536
#47	'individual management':ab,ti,kw	631
#48	(multidimensional NEAR/2 (management OR therapy OR care)):ab,ti,kw	613
#49	(multidisciplinary NEAR/2 (team$ OR intervention$ OR care)):ab,ti,kw	79513
#50	(tailored NEAR/2 (care OR therapy OR rehabilitation)):ab,ti,kw	6767
#51	'primary care':ab,ti,kw	215658
#52	'first-line care':ab,ti,kw	176
#53	((recovery OR comprehensive) NEAR/2 program):ab,ti,kw	7379
#54	((rehabilitation OR management OR assessment) NEAR/2 service):ab,ti,kw	9769
#55	'level of care':ab,ti,kw	8612
#56	#30 OR #31 OR #32 OR #33 OR #34 OR #35 OR #36 OR #37 OR #38 OR #39 OR #40 OR #41 OR #42 OR #43 OR #44 OR #45 OR #46 OR #47 OR #48 OR #49 OR #50 OR #51 OR #52 OR #53 OR #54 OR #55	1164450
#57	#29 OR #56	1987795
#58	#16 AND #57	3679
#59	#58 AND [2014-2024]/py	2759
#60	#59 NOT [medline]/lim	1479
#61	#60 NOT ('conference abstract'/it OR 'conference paper'/it OR 'conference review'/it)	574
#62	#61 AND ('meta-analysis'/exp OR 'meta-analysis' OR 'systematic review'/exp OR 'systematic review')	28
#63	'clinical pathway'/de	10632
#64	'clinical protocol'/exp	129635
#65	'consensus'/exp	108919

#66	'consensus development'/de	30043
#67	'practice guideline'/exp	772866
#68	'guideline'/de	142
#69	'health care planning'/de	117547
#70	'position statement*':ab,ti,kw OR 'policy statement*':ab,ti,kw OR 'practice parameter*':ab,ti,kw OR 'best practice*':ab,ti,kw	75206
#71	standards:ti,kw OR guideline:ti,kw OR guidelines:ti,kw	201165
#72	((practice OR treatment*) NEAR/3 guideline*):ab	93064
#73	cpg:ti,kw OR cpgs:ti,kw	11362
#74	consensus*:ti,kw	49061
#75	((critical OR clinical OR practice) NEAR/3 (path OR paths OR pathway OR pathways OR protocol*)):ab,ti,kw	61935
#76	recommendat*:ti,kw	74471
#77	(care NEAR/3 (standard OR path OR paths OR pathway OR pathways OR map OR maps OR plan OR plans)):ab,ti,kw	180871
#78	(algorithm* NEAR/3 (screening OR examination OR test OR tested OR testing OR assessment* OR diagnosis OR diagnoses OR diagnosed OR diagnosing)):ab,ti,kw	24158
#79	(algorithm* NEAR/3 (pharmacotherap* OR chemotherap* OR chemotreatment* OR therap* OR treatment* OR intervention*)):ab,ti,kw	24682
#80	#63 OR #64 OR #65 OR #66 OR #67 OR #68 OR #69 OR #70 OR #71 OR #72 OR #73 OR #74 OR #75 OR #76 OR #77 OR #78 OR #79	1449450
#81	#61 AND #80	78
#82	#61 NOT (#62 OR #81)	480

Embase		
Date: 19 March 2026		
#1	'chronic fatigue syndrome'/exp	27727
#2	(chronic* NEAR/3 fatigue):ab,ti,kw	16858
#3	(fatigue* NEAR/2 (disorder* OR chronic OR syndrome OR persistent OR severe OR severity)):ab,ti,kw	27832
#4	(fatigue* NEAR/2 (postviral OR 'post viral' OR 'post infection*' OR postinfection* OR 'immune dysfunction*')):ab,ti,kw	354
#5	((myalgic OR 'post infection*' OR postinfection*) NEAR/3 (encephalomyelitis OR encephalopath*)):ab,ti,kw	2716
#6	cfs:ab,ti,kw	16862
#7	cfids:ab,ti,kw OR pvfs:ab,ti,kw	59
#8	(me NEAR/1 seid):ab,ti,kw	4
#9	((chronic NEAR/2 'epstein barr virus'):ab,ti,kw) OR cebv:ab,ti,kw OR caebv:ab,ti,kw OR 'chronic mononucleosis':ab,ti,kw	1068
#10	(('orthostatic intolerance' OR 'postural orthostatic tachycardia syndrome' OR 'postural tachycardia syndrome' OR pots) NEAR/6 (cfs OR 'chronic* fatigue*' OR me OR myalgic OR seid OR 'systemic exertion')):ab,ti,kw	205
#11	'systemic exertion intolerance disease':ab,ti,kw OR seid:ab,ti,kw	139

#12	(('post-exertional' OR postexertional) NEAR/2 malaise):ab,ti,kw	774
#13	'neurasthenic neuroses':ab,ti,kw OR 'epidemic neuromyasthenia':ab,ti,kw OR neurataxia:ab,ti,kw OR neuroasthenia:ab,ti,kw OR neurasthenia:ab,ti,kw	994
#14	'effort syndrome*':ab,ti,kw	78
#15	#2 OR #3 OR #4 OR #5 OR #6 OR #7 OR #8 OR #9 OR #10 OR #11 OR #12 OR #13 OR #14	43614
#16	#1 OR #15	59111
#17	'health care delivery'/de	232562
#18	'integrated health care system'/de	15181
#19	'primary care access'/de	535
#20	'collaborative care team'/de	5462
#21	'health care access'/de	120467
#22	'patient care'/de	417046
#23	'person centered care'/de	8634
#24	'patient care planning'/exp	31804
#25	'health care'/de	153769
#26	'algorithm'/de	475843
#27	'clinical pathway'/de	11863
#28	'triage'/de	14150
#29	#17 OR #18 OR #19 OR #20 OR #21 OR #22 OR #23 OR #24 OR #25 OR #26 OR #27 OR #28	1386368
#30	'access to care':ab,ti,kw	29630
#31	'patient management':ab,ti,kw	48739
#32	'care coordination':ab,ti,kw	11626
#33	'health care delivery':ab,ti,kw	17280
#34	'multidisciplinary care':ab,ti,kw	13618
#35	'continuity of care':ab,ti,kw	16556
#36	'chronic disease management':ab,ti,kw	6371
#37	triage:ab,ti,kw	50019
#38	'stratified care':ab,ti,kw	373
#39	algorithm$:ab,ti,kw	598698
#40	'process of care':ab,ti,kw	3782
#41	'case management plan$':ab,ti,kw	52
#42	'care map$':ab,ti,kw	297
#43	((clinical OR critical OR care OR healthcare OR health OR patient OR referral OR tailored) NEAR/2 (path OR pathway)):ab,ti,kw	35745
#44	(health NEAR/3 (centre$ OR structure OR facility OR facilities)):ab,ti,kw	82298
#45	((assessment OR referral OR specialized OR specialist OR outpatient$) NEAR/2 (centre$ OR clinic$ OR team$)):ab,ti,kw	153993
#46	(structure NEAR/2 (care OR management)):ab,ti,kw	2774

#47	'individual management':ab,ti,kw	707
#48	(multidimensional NEAR/2 (management OR therapy OR care)):ab,ti,kw	764
#49	(multidisciplinary NEAR/2 (team$ OR intervention$ OR care)):ab,ti,kw	97769
#50	(tailored NEAR/2 (care OR therapy OR rehabilitation)):ab,ti,kw	9377
#51	'primary care':ab,ti,kw	246396
#52	'first-line care':ab,ti,kw	218
#53	((recovery OR comprehensive) NEAR/2 program):ab,ti,kw	8664
#54	((rehabilitation OR management OR assessment) NEAR/2 service):ab,ti,kw	11155
#55	'level of care':ab,ti,kw	9879
#56	#30 OR #31 OR #32 OR #33 OR #34 OR #35 OR #36 OR #37 OR #38 OR #39 OR #40 OR #41 OR #42 OR #43 OR #44 OR #45 OR #46 OR #47 OR #48 OR #49 OR #50 OR #51 OR #52 OR #53 OR #54 OR #55	1360158
#57	#29 OR #56	2257334
#58	#16 AND #57	4794
#59	#58 AND [2014-2024]/py	1193
#60	#59 NOT [medline]/lim	480
#61	#60 NOT ('conference abstract'/it OR 'conference paper'/it OR 'conference review'/it)	175
#62	#61 AND ('meta-analysis'/exp OR 'meta-analysis' OR 'systematic review'/exp OR 'systematic review')	11
#63	'clinical pathway'/de	11863
#64	'clinical protocol'/exp	147169
#65	'consensus'/exp	120344
#66	'consensus development'/de	32476
#67	'practice guideline'/exp	879011
#68	'guideline'/de	144
#69	'health care planning'/de	121776
#70	'position statement*':ab,ti,kw OR 'policy statement*':ab,ti,kw OR 'practice parameter*':ab,ti,kw OR 'best practice*':ab,ti,kw	88799
#71	standards:ti,kw OR guideline:ti,kw OR guidelines:ti,kw	220083
#72	((practice OR treatment*) NEAR/3 guideline*):ab	109364
#73	cpg:ti,kw OR cpgs:ti,kw	11975
#74	consensus*:ti,kw	56002
#75	((critical OR clinical OR practice) NEAR/3 (path OR paths OR pathway OR pathways OR protocol*)):ab,ti,kw	77097
#76	recommendat*:ti,kw	82690
#77	(care NEAR/3 (standard OR path OR paths OR pathway OR pathways OR map OR maps OR plan OR plans)):ab,ti,kw	245248
#78	(algorithm* NEAR/3 (screening OR examination OR test OR tested OR testing OR assessment* OR diagnosis OR diagnoses OR diagnosed OR diagnosing)):ab,ti,kw	27662

#79	(algorithm* NEAR/3 (pharmacotherap* OR chemotherap* OR chemotreatment* OR therap* OR treatment* OR intervention*)):ab,ti,kw	28408
#80	#63 OR #64 OR #65 OR #66 OR #67 OR #68 OR #69 OR #70 OR #71 OR #72 OR #73 OR #74 OR #75 OR #76 OR #77 OR #78 OR #79	1661578
#81	#61 AND #80	26
#82	#61 NOT (#62 OR #81)	141

7.5 Psycinfo

PsycInfo		
Date: 26 November 2024		
1	chronic fatigue syndrome/	2220
2	(chronic* adj3 fatigue).ab,ti,id.	3489
3	(fatigue* adj2 (disorder* or chronic or syndrome or persistent or severe or severity)).ti,ab,id.	4677
4	(fatigue* adj2 (postviral or post viral or post infection* or postinfection* or immune dysfunction*)).ti,ab,id.	54
5	((myalgic or post infection* or postinfection*) adj3 (encephalomyelitis or encephalopath*)).ti,ab,id.	428
6	cfs.ab,ti,id.	2439
7	((ME adj CFS) or (CFS adj ME) or CFIDS or PVFS).ti,ab,id.	333
8	((chronic adj2 epstein Barr virus) or CEBV or CAEBV or chronic mononucleosis).ti,ab,id.	15
9	((Orthostatic intolerance or postural orthostatic tachycardia syndrome or postural tachycardia syndrome or POTS) adj6 (CFS or chronic* fatigue* or ME or myalgic or SEID or systemic exertion)).ti,ab,id.	10
10	((CFS adj2 SEID) or (SEID adj2 CFS) or (ME adj2 CFS adj2 SEID) or (ME adj2 SEID) or (SEID adj2 ME)).ti,ab,id.	1
11	(Systemic Exertion Intolerance Disease or SEID).ti,ab,id.	47
12	((Post-exertional or postexertional) adj2 malaise).ti,ab,id.	73
13	(neurasthenic neuroses or epidemic neuromyasthenia or neurataxia or neuroasthenia or neurasthenia).ti,ab,id.	395
14	effort syndrome*.ti,ab,id.	9
15	2 or 3 or 4 or 5 or 6 or 7 or 8 or 9 or 10 or 11 or 12 or 13 or 14	5959
16	1 or 15	5977
17	health care delivery/	21572
18	integrated services/	5988
19	exp health care access/	11292
20	teamwork/	2868
21	patient centered care/	1043
22	exp Patient Care Planning/	9172
23	17 or 18 or 19 or 20 or 21 or 22	49710
24	Access to Care.ab,ti,id.	5468

25	Patient Management.ab,ti,id.	1339
26	Care Coordination.ab,ti,id.	1820
27	Health Care Delivery.ab,ti,id.	5266
28	Multidisciplinary Care.ab,ti,id.	701
29	Continuity of Care.ab,ti,id.	2865
30	Chronic Disease Management.ab,ti,id.	833
31	triage.ab,ti,id.	2095
32	stratified care.ab,ti,id.	30
33	algorithm?.ab,ti,id.	41775
34	"process of care".ab,ti,id.	591
35	case management plan?.ab,ti,id.	25
36	care map?.ab,ti,id.	20
37	((clinical or critical or care or healthcare or health or patient or referral or tailored) adj2 (path or pathway)).ab,ti,id.	2361
38	(health adj3 (centre? or structure or facility or facilities)).ab,ti,id.	8480
39	((assessment or referral or specialized por specialist or outpatient?) adj2 (centre? or clinic? or team?)).ab,ti,id.	11314
40	(structure adj2 (care or management)).ab,ti,id.	693
41	individual management.ab,ti,id.	69
42	(multidimensional adj2 (management or therapy or care)).ab,ti,id.	292
43	(multidisciplinary adj2 (team? or intervention? or care)).ab,ti,id.	6598
44	(tailored adj2 (care or therapy or rehabilitation)).ab,ti,id.	667
45	primary care.ab,ti,id.	37781
46	first-line care.ab,ti,id.	13
47	((recovery or comprehensive) adj2 program).ab,ti,id.	2398
48	((rehabilitation or management or assessment) adj2 service).ab,ti,id.	2834
49	level of care.ab,ti,id.	1709
50	24 or 25 or 26 or 27 or 28 or 29 or 30 or 31 or 32 or 33 or 34 or 35 or 36 or 37 or 38 or 39 or 40 or 41 or 42 or 43 or 44 or 45 or 46 or 47 or 48 or 49	129948
51	23 or 50	168107
52	16 and 51	370
53	limit 52 to yr="2014-2024"	145
54	(systematic review or systematic literature review or systematic scoping review or systematic narrative review or systematic qualitative review or systematic evidence review or systematic quantitative review or systematic meta-review or systematic critical review or systematic mixed studies review or systematic mapping review or systematic cochrane review or "systematic search and review" or systematic integrative review).ti. or systematic review.pt.	42887
55	meta-analys*.ti.	29948
56	health technology assessment.ti.	121
57	HtA.ti.	29
58	54 or 55 or 56 or 57	61406

59	53 and 58	3
60	treatment guidelines/	10164
61	(guideline or practice guideline or consensus development conference or consensus development conference, NIH).pt.	0
62	(position statement* or policy statement* or practice parameter* or best practice*).ti,ab,id.	23811
63	(standards or guideline or guidelines).ti,id.	25462
64	((practice or treatment* or clinical) adj2 guideline*).ab,id.	13636
65	(CPG or CPGs).ti.	142
66	consensus*.ti,id.	4633
67	consensus*.ab. /freq=2	6076
68	((critical or clinical or practice) adj2 (path or paths or pathway or pathways or protocol*)).ti,ab,id.	2703
69	recommendat*.ti,id.	11133
70	(care adj2 (standard or path or paths or pathway or pathways or map or maps or plan or plans)).ti,ab,id.	11566
71	(algorithm* adj2 (screening or examination or test or tested or testing or assessment* or diagnosis or diagnoses or diagnosed or diagnosing)).ti,ab,id.	855
72	(algorithm* adj2 (pharmacotherap* or chemotherap* or chemotreatment* or therap* or treatment* or intervention*)).ti,ab,id.	915
73	61 or 62 or 63 or 64 or 65 or 66 or 67 or 68 or 69 or 70 or 71 or 72	87344
74	60 or 73	90154
75	53 and 74	6
76	53 not (59 or 75)	136

PsycInfo		
Date: March 2026, week 1		
1	chronic fatigue syndrome/	1791
2	(chronic* adj3 fatigue).ab,ti,id.	2809
3	(fatigue* adj2 (disorder* or chronic or syndrome or persistent or severe or severity)).ti,ab,id.	4017
4	(fatigue* adj2 (postviral or post viral or post infection* or postinfection* or immune dysfunction*)).ti,ab,id.	30
5	((myalgic or post infection* or postinfection*) adj3 (encephalomyelitis or encephalopath*)).ti,ab,id.	454
6	cfs.ab,ti,id.	2014
7	((ME adj CFS) or (CFS adj ME) or CFIDS or PVFS).ti,ab,id.	360
8	((chronic adj2 epstein Barr virus) or CEBV or CAEBV or chronic mononucleosis).ti,ab,id.	7
9	((Orthostatic intolerance or postural orthostatic tachycardia syndrome or postural tachycardia syndrome or POTS) adj6 (CFS or chronic* fatigue* or ME or myalgic or SEID or systemic exertion)).ti,ab,id.	11

10	((CFS adj2 SEID) or (SEID adj2 CFS) or (ME adj2 CFS adj2 SEID) or (ME adj2 SEID) or (SEID adj2 ME)).ti,ab,id.	1
11	(Systemic Exertion Intolerance Disease or SEID).ti,ab,id.	42
12	((Post-exertional or postexertional) adj2 malaise).ti,ab,id.	91
13	(neurasthenic neuroses or epidemic neuromyasthenia or neurataxia or neuroasthenia or neurasthenia).ti,ab,id.	210
14	effort syndrome*.ti,ab,id.	5
15	2 or 3 or 4 or 5 or 6 or 7 or 8 or 9 or 10 or 11 or 12 or 13 or 14	5122
16	1 or 15	5138
17	health care delivery/	17353
18	integrated services/	5805
19	exp health care access/	13117
20	teamwork/	2797
21	patient centered care/	1286
22	exp Patient Care Planning/	9809
23	17 or 18 or 19 or 20 or 21 or 22	47878
24	Access to Care.ab,ti,id.	5976
25	Patient Management.ab,ti,id.	1164
26	Care Coordination.ab,ti,id.	1966
27	Health Care Delivery.ab,ti,id.	4646
28	Multidisciplinary Care.ab,ti,id.	794
29	Continuity of Care.ab,ti,id.	2785
30	Chronic Disease Management.ab,ti,id.	899
31	triage.ab,ti,id.	2100
32	stratified care.ab,ti,id.	33
33	algorithm?.ab,ti,id.	41969
34	"process of care".ab,ti,id.	539
35	case management plan?.ab,ti,id.	21
36	care map?.ab,ti,id.	21
37	((clinical or critical or care or healthcare or health or patient or referral or tailored) adj2 (path or pathway)).ab,ti,id.	2544
38	(health adj3 (centre? or structure or facility or facilities)).ab,ti,id.	8124
39	((assessment or referral or specialized por specialist or outpatient?) adj2 (centre? or clinic? or team?)).ab,ti,id.	10443
40	(structure adj2 (care or management)).ab,ti,id.	574
41	individual management.ab,ti,id.	57
42	(multidimensional adj2 (management or therapy or care)).ab,ti,id.	256
43	(multidisciplinary adj2 (team? or intervention? or care)).ab,ti,id.	6360
44	(tailored adj2 (care or therapy or rehabilitation)).ab,ti,id.	801
45	primary care.ab,ti,id.	35573

46	first-line care.ab,ti,id.	14
47	((recovery or comprehensive) adj2 program).ab,ti,id.	1952
48	((rehabilitation or management or assessment) adj2 service).ab,ti,id.	2454
49	level of care.ab,ti,id.	1624
50	24 or 25 or 26 or 27 or 28 or 29 or 30 or 31 or 32 or 33 or 34 or 35 or 36 or 37 or 38 or 39 or 40 or 41 or 42 or 43 or 44 or 45 or 46 or 47 or 48 or 49	125425
51	23 or 50	161943
52	16 and 51	333
53	limit 52 to yr="2014-2024"	153
54	limit 53 to up="20241126-20260309"	9

7.6 Pedro

Pedro		
Date: 26 November 2024		
1	Abstract & Title: "chronic fatigue"	69
Comments: 26 Systematic reviews, 1 Guideline, 42 Others		

Pedro		
Date: 12 March 2026		
1	Abstract & Title: "chronic fatigue"	22
Comments: 22 Systematic reviews		

8 LIST OF EXCLUDED ARTICLES

Author, year,	Reason(s) for exclusion
Araja *et al. (2023)*	Wrong design (narrative review)
Cullinan et al. *(2021)*	Wrong design (survey among Euromene members on knowledge and understanding of ME/CFS among primary care physicians)
Chowdhury, (2026)*	Wrong publication type (opinion letter/comment)
Friedman *et al. (2019)*	Wrong design (narrative review)
Green *et al. (2015)*	Wrong design (consensus report of a workshop)
Hackett *et al. (2015)*	Wrong outcome (no measure of an outcome related to the organisation of care)
Janse *et al. (2019)*	Wrong intervention (limited to mental health care)
La Cour *(2024)*	Wrong design (qualitative analysis of an organisation of care through patient interviews)
Lapp *et al. (2018)*	Wrong design (narrative review)
MacKinnon, *2023*	Wrong design (narrative review)
Menon *et al. (2020)*	Wrong design (narrative review)
Montoya *et al. (2021)*	Wrong design (narrative review)
Nacul *et al. (2021)*	Wrong design (narrative review/expert consensus)
Ryckeghem *et al. (2017)*	Wrong design (qualitative analysis of an organisation of care through patient interviews)
Saunders *(2022)*	Wrong publication type (opinion letter/comment)
Sommerfelt *et al. (2023)*	Wrong design (patient's experiences with healthcare in general)
Strand *et al. (2019)*	Wrong design (survey among Euromene members to map national guidelines and recommendations for ME/CFS)
Tschopp *et al. (2023)*	Wrong design (survey on disease history, treatments, impact of disease in patients with ME/CFS)
Wadman *(2016)*	Wrong publication type (science journalism piece)
*Result from update search strategy (March 2026)	

9 DESCRIPTION OF THE STUDIES ON THE HEALTHCARE SERVICES FOR SEVERELY ILL INDIVIDUALS

Two articles explored the circumstances of **severely affected ME/CFS patients and patients with early onset** (notably, ME/CFS onset was reported before age 15 in 45% of cases).[1, 2] These studies on severe and very severe patients with ME/CFS highlight significant gaps in the organisation and delivery of ME/CFS care in several European countries. They describe fragmented systems, insufficient professional knowledge, and major unmet needs, particularly for severely affected patients. These insights underscore the urgency of developing coherent, patient-centred organsational models for ME/CFS.

Key-elements from studies on the situation of patients with severe and very severe ME/CFS).[1, 2]
Severely ill patients in Denmark and Norway experience profound functional limitations, including being bedridden, unable to speak, and suffering severe symptom exacerbation from minimal exertion or sensory input.
Caregivers frequently provided more than 40 hours per week of direct care. This burden had significant consequences for their employment, finances, and mental wellbeing.
Childhood onset was common, disease burden severe, and support from healthcare and social systems was widely regarded as grossly inadequate.
In Denmark, there are large regional and municipal differences in the allocation of social and healthcare services. Such as municipalities engaged in years-long legal disputes and appeals with patients and caregivers regarding basic support and assistive devices and others provided limited or no support.
The Danish Health Authority recommended establishing outpatient clinics for “functional disorders” in each of the five regions. However, attendance requirements (2-3 hour group sessions, travel to the clinic) are unsuitable for the most severely ill. A small number of specialised hospital beds were planned but not yet in place, and staff training specific to ME/CFS was lacking.
GP care was perceived as inconsistent, with many severely ill patients denied home visits or losing access to their GP. Reasons included disbelief in ME/CFS, disagreement with specialist treatments, or dismissal of symptoms as weight- or lifestyle-related.
Participants described hospital admissions as harmful, due to lack of knowledge about ME/CFS and disregard for severe sensory hypersensitivity, often resulting in worsening of symptoms.
Psychiatric interventions were commonly experienced as negative or inappropriate, contributing to mutual distrust between patients and the healthcare system.
Social services were often burdensome to access, with procedures described as more harmful than helpful.
Severe patients and their caregivers were described as a systematically neglected patient group, receiving less appropriate care and support than any comparable group with similar levels of illness.

10 CONSULTED INTERNATIONAL EXPERTS

10.1 France

- **De Korwin J.D.**, University of Lorraine, Faculty of Medicine; member of Commission des Recommandations, Parcours professionnels, Indicateurs (CRPPI) from Haute Autorité de Santé (HAS)- France ; Emeritus Professor at Internal Medicine Department, University of Lorraine, 34, Cours Léopold, CS 25233, CEDEX F-54052 Nancy, France
- **Retornaz F.**, Unité de soins et de recherche en médecine interne et maladies infectieuses, European Hospital, Marseille, France

10.2 The Netherlands

- **Knoop H.**, Medical Psychology, Amsterdam UMC location University of Amsterdam, The Netherlands, Amsterdam
- **Vermeulen R.C.W.**, CVS/ME Medisch Centrum, Amsterdam, The Netherlands

10.3 Italy

- **Lorusso L.**, Neurology Unit, Neuroscience Department A.S.S.T. Lecco, Merate Hospital, 23807 Merate, Italy

10.4 United Kingdom

- **Townend M.**, Post Viral Fatigue service, Malvern Community Hospital, 185 Worcester Road, Malvern, UK; Deputy Chair of BACME (British Association of Clinicians in ME/CFS).

11 QUESTIONNAIRE ON HEALTHCARE SERVICES FOR INDIVIDUALS WITH ME/CFS

Structural organisation, regulatory framework and governance

- **In your country, are there any structures involved in the management of ME/CFS that are organised and funded by the official health system (health authorities/national plan)?**
  - If so, could you specify the number in the territory and their composition (medical and paramedical specialities)?
- **If this is not the case, are there any reference centres developed by the medical/academic initiative of specialists?**
  - If so, could you specify the number in the territory and their composition (medical and paramedical specialities)?
  - Is treatment at these specialist centres reimbursed?
- **Are there national guidelines for ME/CFS that define diagnostic criteria, types of treatment, and care pathways?**
- **Do existing structures for managing ME/CFS enable patients to receive care that is appropriate to their needs?**

As an ME/CFS specialist, could you describe your centre's experience? We are particularly interested in the following aspects:

- Types of care provided: diagnostic confirmation, treatments (regular or limited-period follow-up)
- Types of medical and paramedical specialities
- Types of treatment offered (at the centre or provided outside the centre)
- Approximate number of patients per year
- Accessibility, waiting list
- Contact with general practitioners

REFERENCES

1. la Cour P. The most severely ill patients with ME/CFS in Denmark. Cogent Public Health. 2024;11(1).

2. Sommerfelt K, Schei T, Angelsen A. Severe and Very Severe Myalgic Encephalopathy/Chronic Fatigue Syndrome ME/CFS in Norway: Symptom Burden and Access to Care. J Clin Med. 2023;12(4).